intersex
NIGERIA
CENTER FOR HEALTHCARE DEVELOPMENT & YOUTH EMPOWERMENT

VISIBILITY, INCLUSION AND RECOGNITION OF INTERSEX PERSONS IN NIGERIA

A Baseline Study on The Public Perception of Intersex Persons and Intersex Genital Mutilation in Nigeria



By Intersex Nigeria (Centre for Healthcare Development and Youth Empowerment)

Contact Information

Intersex Nigeria (Center for Healthcare Development and Youth Empowerment)

Lagos and Abuja, Nigeria

info@intersexnigeria.org

+2349129057794

Visit our website

www.intersexnigeria.org

Social Media

@Intersex_nigeria_

@IntersexNigeria

@IntersexNigeria

Intersex Nigeria

Published by: Intersex Nigeria
Copyright: Intersex Nigeria, 2024

Publication Date: October, 2024



© Intersex Nigeria

ACKNOWLEDGEMENTS

Profound gratitude and appreciation to all those who contributed to the successful completion of this baseline study titled "Visibility, Inclusion, and Recognition of Intersex Persons in Nigeria." This research report is a testament to the dedication and collaboration of numerous individuals and organizations that made this possible. We give thanks to the intersex community in Nigeria, whose willingness to share their experiences, challenges, and aspirations was crucial to the realization of this study. Your resilience and determination call us to continue the fight for liberation, leaving no one behind. We give thanks to the research participants who generously offered their time and insights, allowing us to gain a comprehensive understanding of the issues faced by intersex persons in Nigeria. Your trust and collaboration have contributed to the unearthing of the multifaceted dimensions of this research.

We appreciate the Consultants **Oluwatoyin Adejonwo, PhD** and **Barr. Omolara Oriye** for their incredible work in collating the data and ensuring the accuracy of information and findings of this report. Your dedication to ensuring an inclusive and enabling society for intersex people is admirable.

We appreciate various non-governmental organizations, human rights advocates, and activists who have been tirelessly working to raise awareness about intersex issues in Nigeria. Your dedication to this cause is commendable, and your support in disseminating this research is invaluable. While we aspire more knowledge and insight into the intersex variations landscape, we thank the academic and research community for their work. Your contributions have added depth and credibility to the findings presented in this report. Finally, we appreciate the **Open Society Initiative for West Africa (OSIWA),** whose financial support made this research possible. Your commitment to advancing the rights and well-being of intersex individuals in Nigeria is deeply appreciated.

In conclusion, we appreciate the incredible work of **Web Haptic Intelligence Group.** We thank **David Benson** for his remarkable editorial contributions to this report. And our immense gratitude to **Loé Petit** for thoroughly and painstakingly proofreading and translating the research study from English to French to retain its context and authenticity. We have faith that this study will be a catalyst for positive change, leading to increased visibility, inclusion, and recognition of intersex persons in Nigeria and beyond.

EXECUTIVE SUMMARY

The deeply ingrained classification of humanity into two distinct sexes—male and female—permeates our understanding and organizations of social structures, and this leads to the exclusion and erasure of intersex persons. This binary perspective fails to account for the existence of individuals who do not conform neatly to the accepted categories. This raises a significant concern as they are vulnerable and exposed to severe human rights violations due to this social order. The widely accepted gender dichotomy, along with the corresponding medical, social and intellectual norms, has given rise to commonplace medical and surgical interventions on intersex individuals that are often carried out without the informed consent or proper consultation of the individuals themselves. The culture of secrecy and shame around the bodies of intersex persons allows these practices to persist, underlying the human rights concerns that remain unrecognized and unaddressed, driving a prevailing lack of awareness within Nigerian society regarding intersex individuals.

The knowledge and awareness gap among the general population and the unwillingness of policymakers exacerbate the challenges faced by intersex individuals in Nigeria. Intersex persons are denied agency over their bodies due to the complex interplay between gender norms, medical practices, and societal attitudes. Such a situation is contrary to the principles of individual autonomy and dignity that form the foundation of fundamental human rights. This directly and indirectly perpetuates harmful practices including discrimination, stigmatization, and exclusion. The 'Visibility, Inclusion and Recognition of Intersex Persons in Nigeria' research provides a comprehensive view of the challenges of visibility, inclusion, and recognition, social perception of intersex persons in Nigeria while expanding on the scope, extent and practice of Intersex Genital Mutilation (IGM). In highlighting challenges faced by intersex persons, including societal stigma, discrimination, and violations of their human rights, the report examines the link between social perception and the experiences of intersex persons, including the experiences of parents of intersex children in access to social services including education, justice, employment, and healthcare.

The report sheds light on the often-overlooked intersex identity and access in a world where some progress has been recorded on the frontiers of intersex rights and emphasizes the need for understanding, awareness, and policy strides in addressing the human rights violations experienced by intersex persons in Nigeria. This report concludes that it is instructive for Nigerian society to confront these deeply rooted issues and undertake a concerted effort to increase awareness, understanding, and

inclusion of intersex individuals. This will require challenging traditional gender norms, advocating for comprehensive education and information dissemination, and promoting a culture of respect and inclusion. There must be a collective commitment to dismantling the secrecy and shame that enable the persistence of harmful practices. Policies and legal frameworks must be put in place to protect the rights of intersex individuals, ensuring that they can make informed decisions about their bodies and live free from discrimination and violence.

Ultimately, this report recommends recognizing and affirming the rights of intersex persons as a matter of justice and a crucial step towards building a more inclusive and equitable society. By acknowledging the reality of intersex persons and working to create a world where their rights are respected and upheld, Nigeria will take a significant stride towards achieving human rights for all.

TABLE OF CONTENTS

	Acknowledgments	I
	Executive Summary	II
	Table of Content	IV
	Acronyms	1
1.0	**Introduction**	2
2.0	**Review of Human Rights and Intersex Rights in Nigeria**	4
2.1	Myths, Misconceptions and Perceptions of Intersex in Nigeria	8
2.2	Prevalence, Extent And Scope of Intersex Genital Mutilation In Nigeria	11
2.2.1	What Is Intersex Genital Mutilation (IGM)?	11
2.2.2	Prevalence, Extent And Scope of IGM In Nigeria	12
2.3.	Scope of Study	13
3.	**Methodology**	14
4.	**Study Outcome**	15
4.1	Understanding of the Term Intersex	18
4.1.1	Perception of Intersex	20
4.1.2	Situation of Intersex in Nigeria	24
4.2.	Public Perception Of Intersex Genital Mutilation (IGM)	30
4.3.	Legal Framework Protecting Intersex Persons from IGM	31
4.3.1	Awareness of Challenges Faced by Intersex Persons in Nigeria	32
4.4.	Perception of Intersex Genital Mutilation in Nigeria	33
4.4.1.	Personal Knowledge of IGM Survivors	34
4.4.2	Perception of Legal Framework Protecting Intersex from IGM	36
5.	**Conclusion**	37
5.1.	**Recommendations**	38
	References	41

ACRONYMS

DSD	-	Disorder of Sexual Development
UDHR	-	Universal Declaration of Human Rights
ICCPR	-	International Covenant for Civil and Political Rights
ICESCR	-	International Covenant for Economic, Social and Cultural Rights
ACHPR	-	African Commission on Human and People's Rights
OHCHR	-	Office of the High Commission on Human Rights
KNCHR	-	Kenya National Commission on Human Rights
SDA	-	Sex Discrimination Act
NHRC	-	National Human Rights Commission of Nigeria
IGM	-	Intersex Genital Mutilation
NCE	-	National Council on Education
NERDC	-	Nigerian Educational Research and Development Council

INTRODUCTION

In the new landscape of sex and gender, a world where trans and gay rights have made some strides, intersex is perhaps the last taboo. 'Intersex' is an umbrella term that describes bodies that fall outside the male/female binary. It describes people who are born with natural sex characteristics, including genitals, gonads, and chromosome patterns, that do not fit typical binary notions of male or female bodies (Carpenter, 2020). Intersex persons have been referred to as 'hermaphrodites' or persons with 'disorders of sexual development' (DSD). Both terms are stigmatizing and infer that it is something that must be corrected.



Intersex variations present in different ways including genitals or internal sex organs that fall outside the male/female categories, such as a person with both ovarian and testicular tissues. Other intersex people have combinations of chromosomes that are different from XY (usually associated with males) and XX (usually associated with females), like XXY. Variations of intersex traits may be visible at birth while others only manifest during puberty. It is important to note that intersex is a term for a diverse range of different traits that can be determined prenatally, at birth, during puberty and at other times. These traits include congenital adrenal hyperplasia, Klinefelter syndrome, hypospadias, and androgen insensitivity syndrome.

There is no conclusive data on the number of intersex persons globally or indeed within the African region, however, experts estimate that intersex persons constitute 0.05 to 1.7% of the total global population. Kenya is the first African nation and

probably one of the only countries in the world to incorporate the intersex category in their national census. Its 2019 census estimates that 1,524 people, about 0.003 per cent of the population, are intersex.

Gender binary, also known as gender binarism, is the classification of gender into two distinct, opposite forms of masculine and feminine, whether by social system, cultural belief, or both simultaneously. Most cultures operate under a gender binary and prescribe to having two genders boys/men and girls/women. Herein lies the root of the challenge, stigmatization, and unfounded perception of intersex persons by society.

There is a general lack of awareness and apathy towards intersex persons in Nigeria from the government, policy and lawmakers, religious organizations, the medical professionals, families, and the society in general. Even though intersex persons live different kinds of lives, they are 2 strongly shaped by everyday discriminatory experiences which deny them professional and personal development as well as social participation. Even when they have the education and vocational skills to earn a livelihood, they are prevented from exploiting their full potential because of who they are.

Without access to important and necessary information needed to make informed decisions, parents are led to think that surgical mutilation in childhood is the best care option for their intersex child. This unnecessary medical intervention, surgeries, and other discriminatory practices are perpetuated on intersex children and persons, sometimes with the active support of the government. In 2019, the Kano State government sponsored forced corrective surgeries on intersex persons. The Governor of the State at the time, stated that the state would "fish out" more intersex persons to undergo these invasive surgeries.

Generally, the experience of intersex persons in Nigeria varies and it is largely influenced by their location. Research indicates that living in one of the cosmopolitan cities like Lagos, Abuja or Port Harcourt is socio-economically preferred. Living in Lagos can provide relief from their growing-up experience and the possible trauma of adolescence. It can give intersex people more fulfilment because of the diversity of people, cultures, and beliefs. Intersex persons growing up and living in Northern Nigeria face a different reality.

2. REVIEW OF HUMAN RIGHTS AND INTERSEX RIGHTS IN NIGERIA



Every person has inherent dignity and value, and to acknowledge this fundamental worth, we recognize and respect their human rights. Human rights are rights we have because we exist as human beings. They are intrinsic to all, regardless of sex, national or ethnic origin, color, religion, language, or any other status. They are a set of principles concerned with equality and fairness, ranging from the most fundamental the right to life, the right to a fair trial, freedom from torture and other cruel and inhuman treatment, freedom of speech, and freedom of religion to those that make life worth living the right to health, education, safe environment, food, work, and an adequate standard of living.

International and regional human rights instruments and national constitution's guarantee these rights, starting with the Universal Declaration of Human Rights (UDHR), adopted by the UN General Assembly in 1948, the International Covenant for Civil and Political Rights (ICCPR, 1966), and the International Covenant for Economic, Social and Cultural Rights (ICESCR, 1966). The UDHR, together with the ICCPR and the ICESCR make up the International Bill of Rights. At the regional level, there is the African Commission on Human and People's Rights (ACHPR) which passed resolution 552 that calls on the promotion and protection of intersex rights in Africa, an international instrument regarding the human rights and freedoms of persons in Africa which provides and guarantees their fundamental civil, political, economic, social, and cultural rights.

In recent years, there has been greater recognition and awareness of specific human rights abuses and violations against intersex persons. These include violations of their rights to be free from torture and ill-treatment; rights to health and bodily integrity, rights to equality and non-discrimination; rights to protection from forced and coercive medical interventions, infanticide, restrictions on the exercise of legal capacity and access to reparation and justice; rights to education, sports, employment, and services.

The Constitution of the Federal Republic of Nigeria 1999 provides a starting point for the promotion and protection of intersex persons in Nigeria. Human rights are constitutionally guaranteed under Chapter IV of the 1999 Constitution as amended. These rights are referred to as inalienable rights and include the right to life (Section 33), right to dignity (Section 34), right to personal liberty (Section 35), right to fair hearing (section 36), right to privacy and family life (section 37), right to freedom of thought conscience and religion (section 38), right to freedom of expression and press (section 39), right to peaceful assembly and association (section 40), right to freedom of movement (section 41), and right to freedom from discrimination.

Further to the rights guaranteed in Chapter IV of the 1999 Constitution, The Fundamental Right (Enforcement Procedures) Rules 2009 made under Section 46 (3) of the Constitution provides the mechanisms for the enforcement of fundamental human rights. It provides the practice and procedure for the enforcement of Human Rights in Nigeria. The Constitution provides that any person who alleges that any of the fundamental rights has been or is likely to be infringed may apply to the Court for redress.

The combination of misconceptions, discrimination, social and religious harmful practices, stigma, taboos, and lack of non-pathologizing information perpetuate human rights violations against intersex persons in Nigeria. These violations prevent intersex people from speaking out on these harmful practices and violations, accessing social services such as healthcare, and information.

Intersex persons and their families are also unable to access helpful information, peer support, and legal services leading to further invisibility within their society.
Around the world, only a handful of countries have implemented measures to prevent and address these abuses, and according to the United Nations Office of the High Commission on Human Rights (OHCHR), the effectiveness of existing measures is yet to be fully documented. In 2005, South Africa included 'intersex' within a broad definition of sex, in the Promotion of Equality and Prevention of Unfair Discrimination Act, 2000 (Equality Act 2000). The Equality Act is a comprehensive anti-discrimination law which forbids hate speech and harassment as well as prohibits

unfair discrimination by the government, private organizations, and individuals. Kenya National Commission on Human Rights (KNCHR) under Article 59 of the Constitution, 2010 and Section 8 of the KNCHR Act, 2011 has the core mandate of ensuring the promotion and protection of human rights in Kenya including safeguarding the rights of intersex persons. Article 27 of the Kenya Constitution provides safeguarding and protections against discrimination of persons based on grounds which include sex.

The Kenyan Constitutional Court has also reaffirmed that intersex persons have the same human rights and fundamental freedoms under the Bill of Rights as any other person. In 2013, Australia included the attribute of 'intersex status' within the Sex Discrimination Act 1984 (SDA). The SDA makes it unlawful to discriminate against a person because of their sex, gender identity, intersex status, sexual orientation, marital or relationship status. Malta's Gender Identity, Gender Expression and Sex Characteristics Act, 2015 prohibits surgery and treatment on the sex characteristics of minors without their informed consent and prohibits discrimination on the grounds of sex characteristics. Malta has also implemented an anti-discrimination policy in schools.

Intersex human rights organizations play a vital role in providing support to intersex people and their families, including providing information, educating, creating awareness, and addressing stigmatization and discrimination. They also document and collect evidence of human rights violations. Government support is critical, especially in the areas of policies and regulations and ensuring easy access to social services and amenities without fear of stigmatization and discrimination.

The National Human Rights Commission of Nigeria (NHRC), affiliated with the African Commission on Human and Peoples' Rights, was established by the National Human Rights Commission Act of 1995 to promote, and protect all human rights in Nigeria. NHRC was established in line with Resolution 48/134 of the United Nations General Assembly which enjoins all member states to establish independent National Institutions for the promotion, protection, and enforcement of human rights. The Commission serves as an extra-judicial mechanism for the respect and enjoyment of human rights. It also provides avenues for public enlightenment, research, and dialogue to raise awareness of human rights issues.

The NHRC is mandated to address all matters relating to the promotion and protection of human rights as guaranteed by the Constitution of the Federal Republic of Nigeria, the United Nations Charter and the Universal Declaration on Human Rights, the International Convention on Civil and Political Rights, the International Convention on the Elimination of all forms of Racial Discrimination, the International Convention on Economic, Social and Cultural Rights, the Convention

on the Elimination of all forms of Discrimination Against Women, the Convention on the Rights of the Child, the African Charter on Human and Peoples' Rights and other international and regional instruments on human rights to which Nigeria is a party.

The recognition and promotion of the right of intersex persons fall under the mandate of NHRC, especially through its complaints and other protective mechanisms. Unfortunately, intersex persons in Nigeria have not been positioned to fully employ this human rights mechanism. While the NHRC is aware of the stigma, discrimination and human rights violations faced by intersex persons in Nigeria, the mechanism has not made any stride in the specific protection of the rights of intersex persons. The key reasons highlighted include the failure on the part of affected persons to make individual reports of discrimination, violence, and stigmatization to the NHRC.

The Commission recognizes IGM, the right to education, and the right to privacy, access to healthcare and freedom from discrimination as some of the most common human rights violations against Intersex persons. However, there are legal gaps. While identifying the gaps in the current legislative framework for the protection of intersex persons in Nigeria, the NHRC affirms that there are no intersex persons-specific regulations, policies, or guidelines and this constitutes a challenge in the protection and promotion of human rights of intersex persons. Medical ethics and rules should guide the interactions of intersex persons with doctors and other medical staff who have access to them. While the commission believes that intersex persons are disproportionately affected by medical negligence and other serious human rights violations, the lack of key data on these happenings has been missing from their understanding and scope of work in the protection and promotion of the rights of intersex persons in Nigeria. While the Child Rights Act 2003 recognizes Female genital mutilation as a crime and a violation of the bodily autonomy of children, intersex children are left unprotected by this law.

In terms of identifying legislative gaps, it's challenging to pinpoint specific issues without an established legal framework in place to address intersex rights and concerns. The absence of such legislation makes it difficult to identify gaps or shortcomings. Therefore, the priority should be on creating a comprehensive legal framework that addresses the rights and needs of intersex individuals in Nigeria.
To enable the NHRC to focus on the promotion of intersex rights, there is a need for reliable and regularly updated data. There are currently no clear statistics and figures regarding the challenges faced by intersex people in Nigeria. One of the mandates of the NHRC is to conduct studies and research into emerging fields of human rights. The NHRC must deploy resources for this purpose.

There is a need for a comprehensive sensitization effort aimed at promoting awareness and understanding of intersex identity and variations. It is important to acknowledge that the existing information is not accurate and there is a need for accurate and accessible information on intersex variations in Nigeria. To advance the principles of equality and non-discrimination, the NHRC believes that sensitization programs focusing on key institutions such as schools and the healthcare and legal infrastructure are key. Anti-discrimination laws specific to intersex persons are pertinent because they are designed to protect against both individual discrimination (committed by individuals) and structural discrimination (arising from policies or procedures that disadvantage certain groups).

2.1. MYTHS, MISCONCEPTIONS AND PERCEPTIONS OF INTERSEX PERSONS IN NIGERIA



There is insufficient research that expounds on Nigerian beliefs and perceptions about people born with innate variations in their sex characteristics (intersex people). There is even less literature on how these beliefs affect the legal and policy landscape in the country. This part will focus on analyzing a social perception survey utilizing available limited material. The aim is to construct a comprehensive understanding of Nigeria's social and cultural context concerning intersex persons.

In some Nigerian cultures prior to the 1700s, it was believed that intersex persons just like twin babies were a bad omen or a curse and were killed at birth Children born with intersex variations were deemed dangerous and these societies at the time considered it morally justifiable to end their lives at birth to prevent potentially catastrophic consequences. In certain instances, mothers of intersex children were denied the opportunity to hold their newborns before they were taken away, and any discussion

of the matter was strictly forbidden. A study conducted in South Africa revealed that one participant described intersex individuals as being perceived within the African context as possessing malevolent entities requiring exorcism It is understandable, therefore, that the birth of an intersex child in this period was emotionally traumatizing for parents because the child's sexual ambiguity was seen as a violation of a social-cultural worldview and such parents are sworn to a cult of secrecy and shame

Parents of intersex children typically do not disclose their intersex status to anyone either within or without the family because they are either embarrassed or ashamed or feel like protecting an intersex child is wrong. Baqutayan alluded to the fact that these parents and families use silence and concealment as strategies to avoid social conflict, losses, punishments, and sanctions for their intersex children. The normative and moral rules that govern these processes are not only related to the bodies but are also present in the various social spaces in which they circulate. They went on to identify two types of stigma in this case. Visible stigma is felt by people who have a perceptible characteristic and thus, the major concern is centered on the manner of dealing with the tension created by interaction with society, and invisible stigma only becomes real when the attribute is revealed, directly or indirectly.

Modern medicine and technological advancements have been both positive and negative. It has pathologized intersex variations and at the same time, afforded individuals with these variations the opportunity of having a sex reassignment and sometimes gender-affirming surgeries. The diagnostic and therapeutic procedures, whatever shape or form they take, mostly involves uninformed and coerced medicalization as well as trying to adjust their bodies to the social norms of masculinity and femininity. Biological characteristics a child is born with have a considerable influence on how they are socialized and expected to behave. Intersex individuals are one of the most vulnerable populations in society with very limited research on them. On the occasion that parents think surgical mutilation in childhood is the best option for their intersex child in order to raise them as either male or female, evidence shows that as the children grow older, they notice other body variations and start suffering complications and lose confidence in themselves.

Nigeria remains largely unaware of the reality of intersex people. As established above, the social perception of intersex persons in Nigeria is largely linked to ideas of birth anomalies, shame, and secrecy. The insistence on sex classification of human beings into two categories is ingrained and social systems are built around this classification. People who do not fit neatly into these two sex categories are often exposed to human rights violations. Among them, intersex people are especially vulnerable and are exposed to discrimination based on their perceived or real gender identity and sex characteristics.

Article 2 of the African Charter stipulates that every person is entitled to enjoy the

rights and freedoms recognized and ensured within the Charter, irrespective of distinctions such as race, ethnic group, color, sex, language, religion, political or other opinions, national and social origin, wealth, birth, or any other status. Article 3 ensures that every individual is to be considered equal before the law and is entitled to receive equal protection under the law. While the African Commission's legal interpretations of these articles are somewhat restricted, the Commission affirmed this principle in the case of Purohit and Another vs The Gambia:

"Articles 2 and 3 of the African Charter form the anti-discrimination and equal protection provisions of the African Charter. Article 2 lays down a principle that is essential to the spirit of the African Charter and is therefore necessary for eradicating discrimination in all its guises, while article 3 is important because it guarantees fair and just treatment of individuals within a legal system of a given country. Their provisions cannot be derogated and therefore must be respected in all circumstances for anyone to enjoy all the other rights provided for under the African Charter."

Resolution 552 *The African Commission on Human and Peoples' Rights (the Commission), meeting at its 74th Ordinary Session in 2023:*

> *Recalling its mandate to promote and protect human and peoples' rights in Africa under Article 45 of the African Charter on Human and Peoples' Rights (the African Charter); Article 21 of the African Charter on the Rights and Welfare of the Child prohibit harmful social and cultural practices. Recognizing that intersex persons, who are born naturally with a chromosomal abnormality and reproductive or sexual anatomy that does not appear to fit the typical definitions of female or male, exist in all African societies. Recognizing that intersex is an inherent handicap at birth and that it should not be considered a taboo in all African societies;*
>
> *Recognizing also that non-consensual and unnecessary surgical and other genital normalization procedures performed on intersex persons, in a medical or other setting, may cause them lifelong physical and psychological suffering, permanent sterility, incontinence, and loss of sexual pleasure.*
>
> *Further recognizing that the above-mentioned non-consensual and unnecessary surgical and other genital normalization procedures have irreversible consequences similar to genital mutilation and can be considered as such;*
>
> *Concerned about human rights violations against intersex persons which include, but are not limited to, rejection in society; infanticide and abandonment of children; lack of proper legal recognition and administrative processes that prevent intersex persons from acquiring or altering identity documents; unfair discrimination in schools, health facilities, competitive sports, work; access to public services; and detention;*
>
> *Further concerned about the invisibility and lack of awareness and sensitivity to the plight and situation of intersex persons in African communities.*

2.2 PREVALENCE, EXTENT AND SCOPE OF INTERSEX GENITAL MUTILATION IN NIGERIA

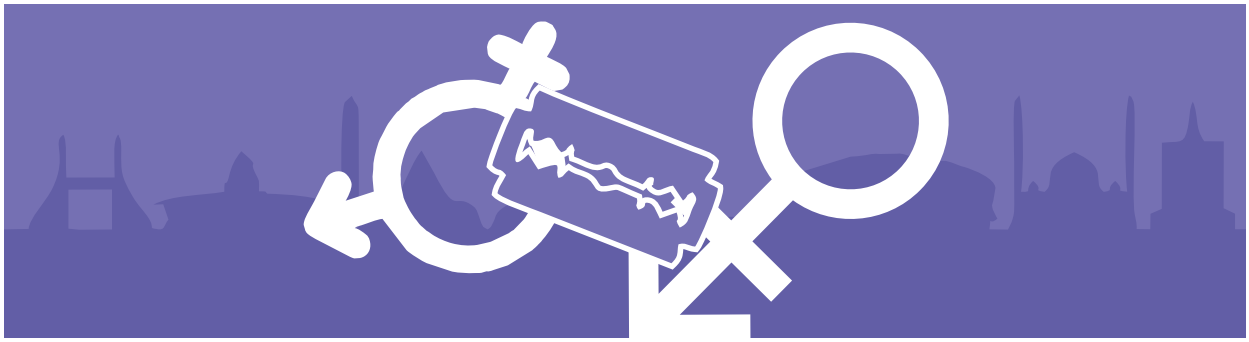
2.2.1 WHAT IS INTERSEX GENITAL MUTILATION (IGM)?



IGM involves surgical interventions for 'sexual normalization', accompanied by hormonal treatments and behavioral instructions according to the assigned sex. Surgeries are performed so that external genitalia would align with the gender the child would be raised in, supposing that they would develop normally. The adjustment of intersex bodies, which in most cases is a form of sex reassignment treatment, was first developed by John Money in a famous experimental case study involving identical twin boys, neither of whom was born with an intersex condition. Money recommended that one of the identical twin boys who had lost his penis because of a circumcision accident be surgically turned into a girl (Joan) and raised as such. Joan grew to love wearing dresses and having her hair done and Money proudly proclaimed the sex reassignment a success. According to Fausto-Sterling, since the 1950s, the medical treatment for intersex conditions has been surgical sex reassignment or hormonal intervention, which is performed to modify ambiguous genitalia and other sex characteristics.

It is believed that modern medicine and technological advancements has afforded individuals with intersexual condition the opportunity of having a sex reassignment to surgically adjust their bodies to fit the societal norms of male and female. This irreversible gender assignment (sometimes non-consensual) often results in several complications afterwards including infertility, severe pain and psychological suffering,

lack of sexual sensation, scarring, removal of gonads leading to sterilisation and life-long hormone treatments as well as psychological trauma for survivors.



The consequences of IGM are an often-irreversible gender assignment. Parents of children with intersex traits often face pressure to agree to such surgeries or treatments on their children. They are rarely informed about alternatives or about the potential negative consequences of the procedures, which are routinely performed despite a lack of medical indication of necessity or urgency. The rationale for these is frequently based on social prejudice, stigma associated with intersex bodies and administrative requirements to assign sex at the moment of birth registration.

Children are not able to make complex decisions for themselves and without the proper guidance and knowledge, parents of intersex children are not well informed or better equipped to take these life altering decisions on behalf of the child, themselves, and the family. Universally, the law presumes that parents act in the best interest of their children, and, thus, are afforded a great deal of latitude in making medical decisions for their children. Parents, therefore, are legally permitted in Nigeria, as is the case in countries such as the United States, United Kingdom etc., to consent to their child's genital surgery in infancy.

Profound negative impacts of these often-irreversible procedures have been reported, including permanent infertility, incontinence, loss of sexual sensation, causing life-long pain and severe psychological suffering, including depression and shame linked to attempts to hide and erase intersex traits. In many cases intersex people do not even have access to their own medical records or original birth certificates.

2.3. SCOPE OF STUDY



The scope of this study encompasses a comprehensive examination of the challenges faced by intersex individuals in Nigeria, addressing key aspects such as societal perceptions, medical practices, and human rights considerations. Through a literature review, key informant interviews, and a social perception survey, the study aims to elucidate the prevalence, socio-cultural contexts, and lived experiences of intersex persons while highlighting prevalent myths, misconceptions, and harmful practices such as Intersex Genital Mutilation (IGM). It also explores the impact of societal stigma and discrimination on intersex individuals' access to education, healthcare, employment, and justice. Additionally, the study proposes policy and advocacy recommendations aimed at increasing awareness, understanding, and inclusion of intersex individuals, challenging traditional gender norms, and promoting respect for their rights and autonomy, ultimately advocating for a more equitable and inclusive society in Nigeria.

3. METHODOLOGY



The research uses both quantitative and qualitative data to reach in-depth conclusions on the social positions of intersex persons and the prevalence of IGM in Nigeria. The qualitative aspects of the data draw insights from an extensive baseline study that includes: literature reviews, in-depth key informant interviews with 5 intersex respondents and 3 parents of intersex children and focus group discussions with 15 intersex persons, highlighting the unique personal experiences of intersex persons and parents.

In gathering quantitative data, the research employed a poll aimed at identifying the social perception of intersex persons in Nigeria and assessing the prevalence of Intersex Genital Mutilation (IGM). The poll was designed to explore diverse demographics to construct a comprehensive understanding of the state of intersex visibility, inclusion, and recognition in Nigeria. Specifically, the quantitative poll surveyed 1000 respondents, comprising 505 males and 495 females. It was conducted across the six geopolitical zones of Nigeria, ensuring representation from various regions, and the respondents were stratified by gender, religious beliefs, and age to provide a clear and relatable report. This approach allowed for a nuanced analysis of public attitudes and perceptions towards intersex identities, as well as the prevalence and impact of IGM practices within different segments of Nigerian society.

4. STUDY OUTCOME



To examine societal attitudes towards intersex persons in Nigeria, a comprehensive perception survey was conducted across all six geopolitical zones of the country. This survey, involving 1000 participants, aimed to uncover prevailing attitudes towards intersex individuals, as well as to address myths, misconceptions, and perceptions regarding their fundamental human rights.

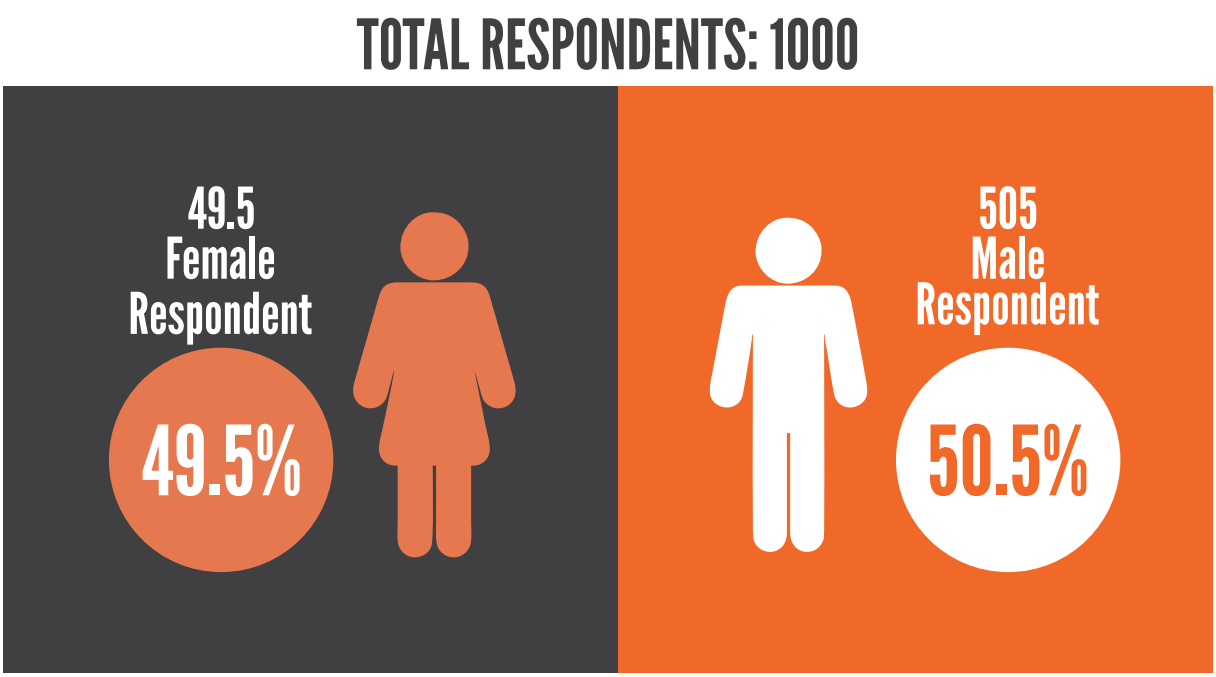


Figure 1. Sex distribution of respondent

Diverse perspectives were collected from individuals representing various backgrounds and regions, with 49.5% of respondents identifying as women and 50.5% as men, as shown in Figure 1. Through this analysis, the study seeks to illuminate current perceptions surrounding intersex individuals, fostering a more inclusive and understanding society for all.

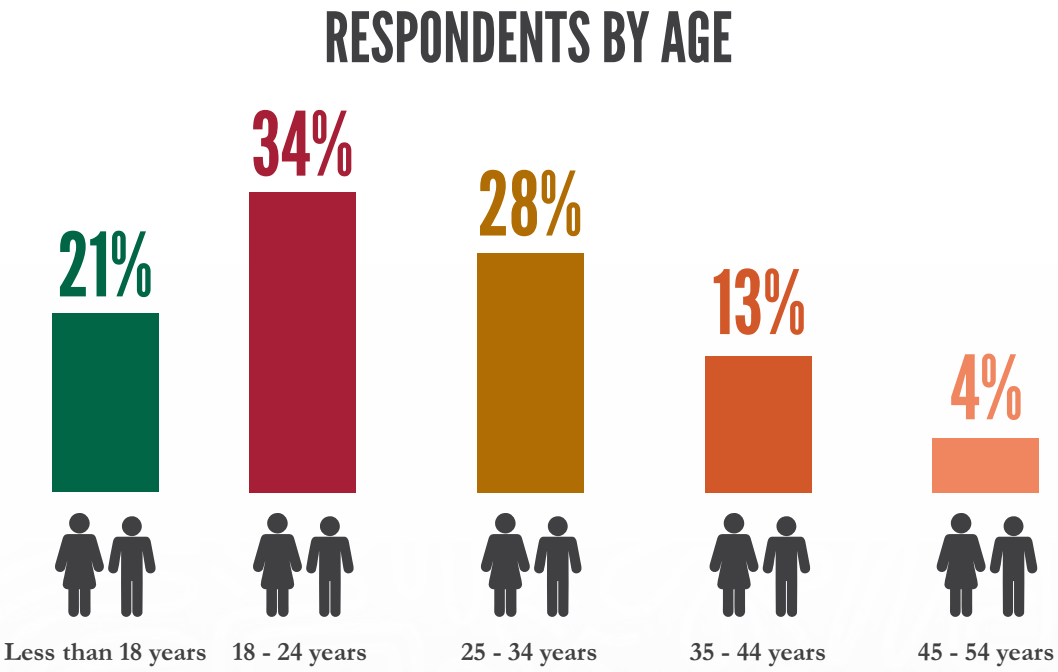


Figure 2. Responses distributed by age.

Figure 2 illustrates the distribution of respondents across various age groups. 21% were below 18 years old, representing the perspectives of the country's youth, crucial in shaping future societal attitudes towards intersex individuals. Additionally, 34% were aged between 18 and 24, reflecting the views of young adults considered pivotal in driving social change. Furthermore, 28% fell within the age range of 25 to 34, and 13% were aged 35 to 44, representing individuals in their prime working and family-building years. Finally, 4% were between 45 to 54 years old, offering insights from more experienced segments of society.

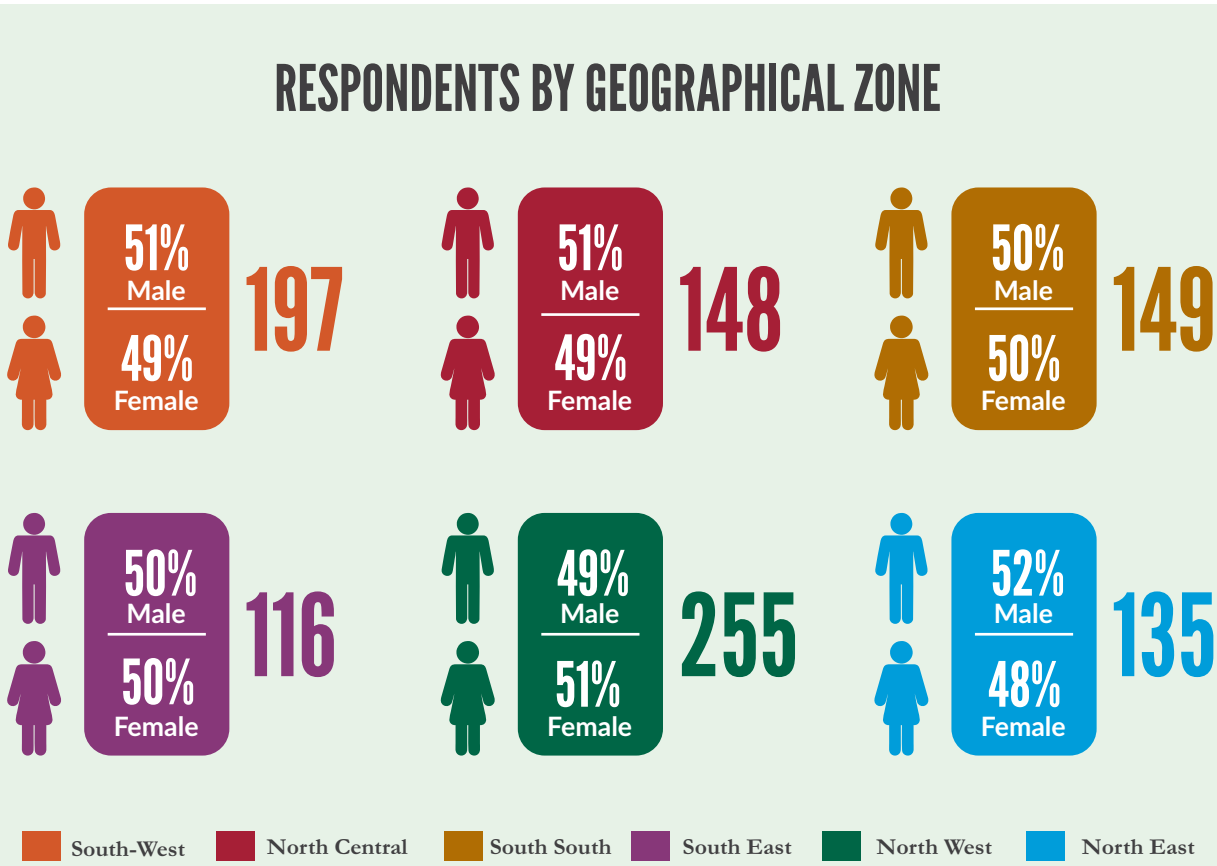


Figure 3. Distribution of respondents by zones

Figure 3 illustrates the breakdown of respondents by their respective geopolitical zones, providing insights into regional variations. In the North Central region, 51% of respondents were male, and 48% were female. Moving to the Northeast, there were 59% male respondents and 41% female respondents, indicating a higher male representation. In contrast, the Northwest had 40% male and 60% female respondents, showing a higher female representation. In the Southeast, 41% were male and 59% were female respondents, while the South-South had slightly more males at 42% and 58% females. Finally, in the Southwest, the majority were male

respondents at 52%, with 48% being female. This demographic distribution across geopolitical zones provides an understanding of attitudes towards intersex persons, reflecting diverse regional dynamics within Nigeria.

4.1 Understanding of the Term Intersex

The survey results regarding respondents' understanding of the term "intersex" show a nearly equal split, with 48% saying they have heard of it and 52% indicating they haven't. This suggests that many people in Nigeria aren't familiar with intersex.

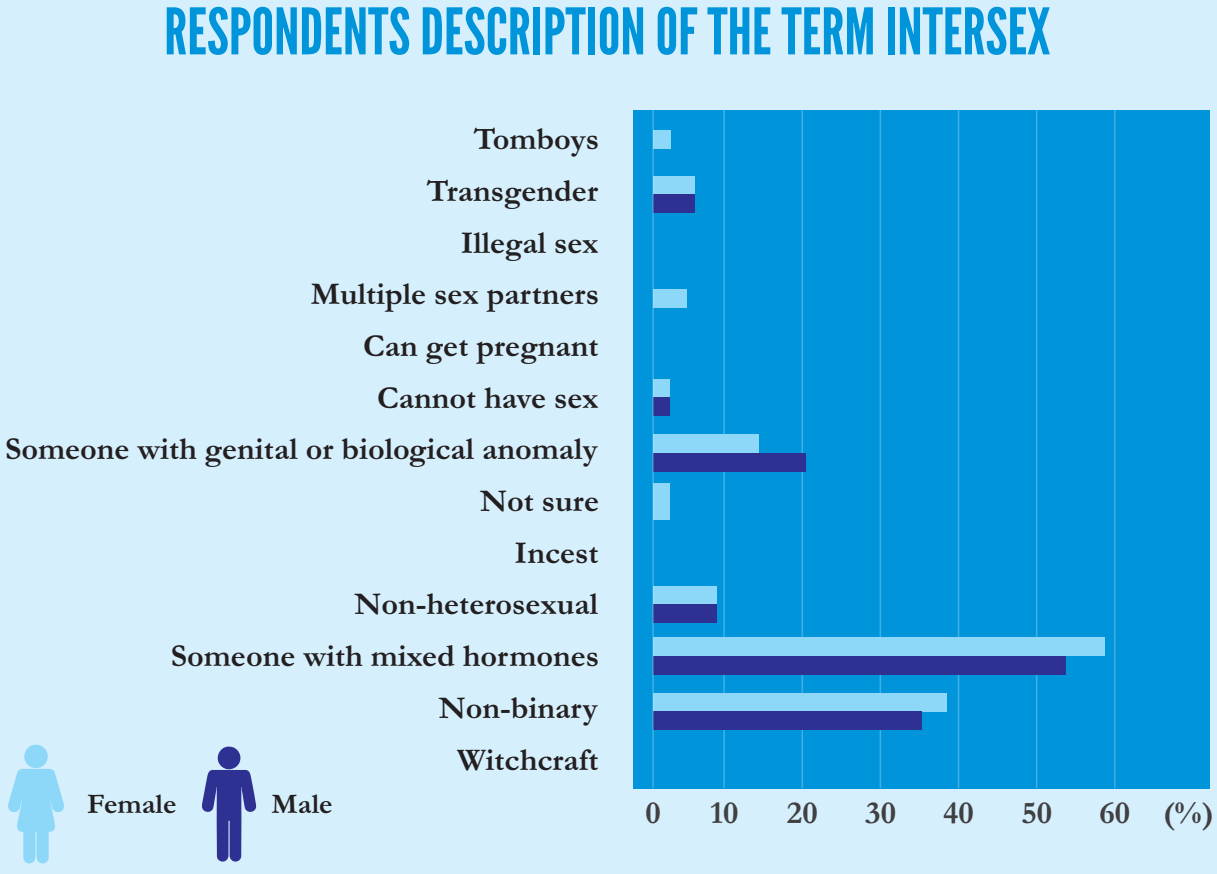


Figure 4. Respondents' description of the term intersex.

Figure 4. depicts the survey's findings on how respondents understand intersex variations, revealing a range of perceptions. The majority (54%) believe intersex individuals have mixed hormones, showing recognition of the biological

aspect. 30% associate intersex variations with non-binary individuals, indicating awareness of gender identity complexities. However, 5% mistakenly link it with non-heterosexual individuals, showing a need to address misconceptions about sexual orientation and intersex. Alarmingly, 1% think intersex individuals can't engage in sexual activity, highlighting the importance of dispelling myths about their capabilities. 2% associate intersex variations with being transgender, indicating confusion between these identities.

i. Are People Born Intersex?



The results show a strong consensus among respondents, with 83% agreeing that people are born intersex, indicating widespread recognition of intersex variations as inherent from birth. Only 6% disagreed, suggesting broad acknowledgment of intersex biological nature. However, 10% expressed uncertainty. Interestingly, both men (82%) and women (85%) largely agree that individuals are born intersex, demonstrating a consensus across genders. Those uncertain (10% each) may benefit from further education on intersex.[14]

ii. Information Source About Intersex Variations



Figure 5 below illustrates the various sources from which respondents obtained information about intersex variations. The primary source, cited by 52% of participants, was the media, including news outlets, television, and the internet, highlighting its significant influence on shaping public perceptions. Media outlets must portray intersex individuals accurately to avoid perpetuating misconceptions or stereotypes. Additionally, 19% reported acquiring information from other people, 15% received information through friends, and 8% through family, emphasizing the influential role of personal connections in shaping understanding and awareness.

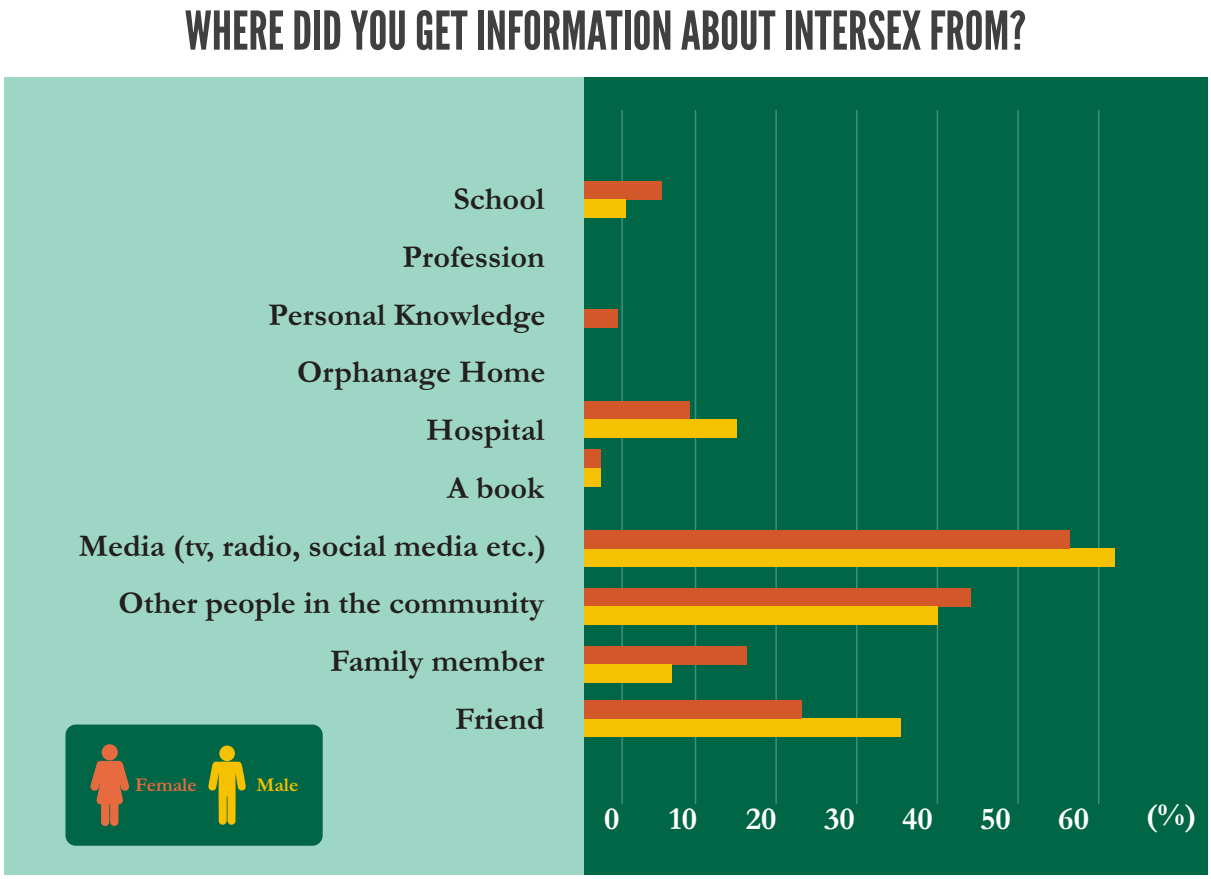


Figure 5: Responses of information sources about intersex

4.1.1 Perception on Intersex

The Figure 6 results below reveal a clear majority opinion, with 78% of participants stating that intersex is not a medical condition, indicating widespread understanding that intersex is a natural aspect of human diversity rather than a disorder. This consensus holds consistent across both genders, with 77% of men and 78% of women sharing this perspective, suggesting gender does not significantly influence this perception. Overall, these findings suggest a prevailing understanding and acceptance that intersex is not a medical condition, reflecting a more inclusive and compassionate perspective towards intersex individuals.

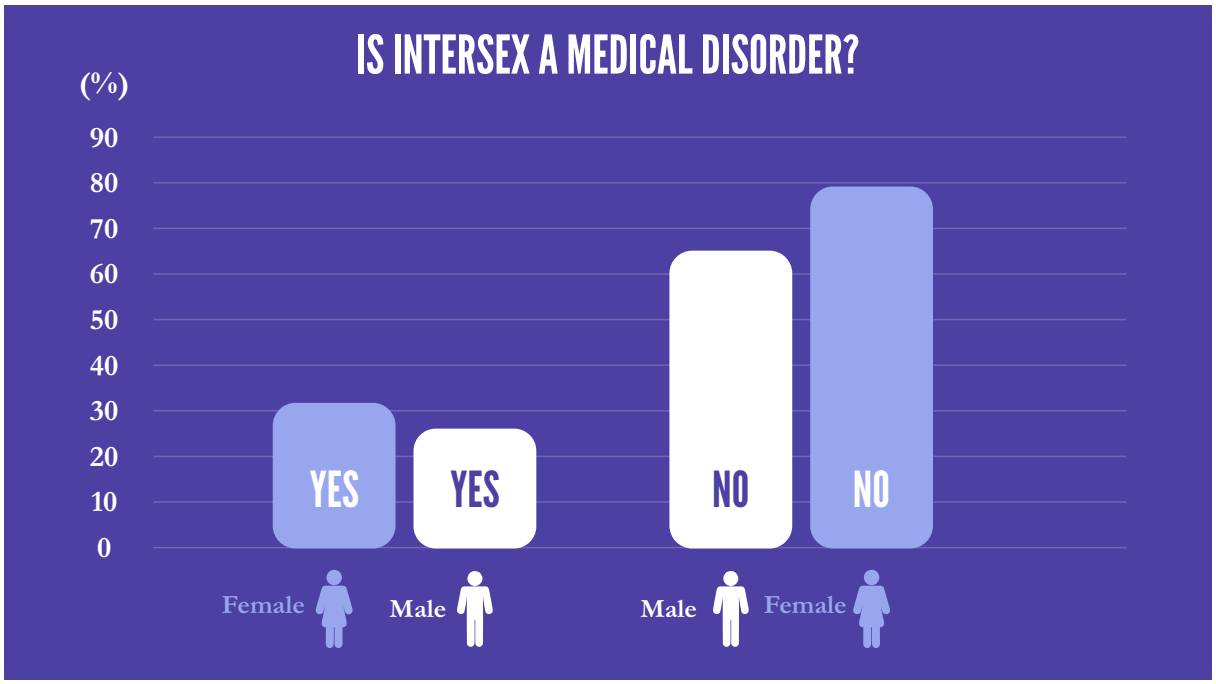


Figure 6. Responses on views of Intersex as a medical condition

i. Family perception of Intersex Individual

Figure 7i. shows the survey results regarding the acceptance of intersex family members revealing a predominantly positive attitude across different age groups and genders. Overall, 76% of respondents expressed willingness to accept an intersex family member. Both male and female respondents demonstrated high acceptance rates, with 78% of men and 74% of women agreeing. While age differences were observed, with younger respondents displaying higher acceptance rates, the data suggests that age may influence perception. Younger generations showed more acceptance, while older generations exhibited greater uncertainty or resistance.

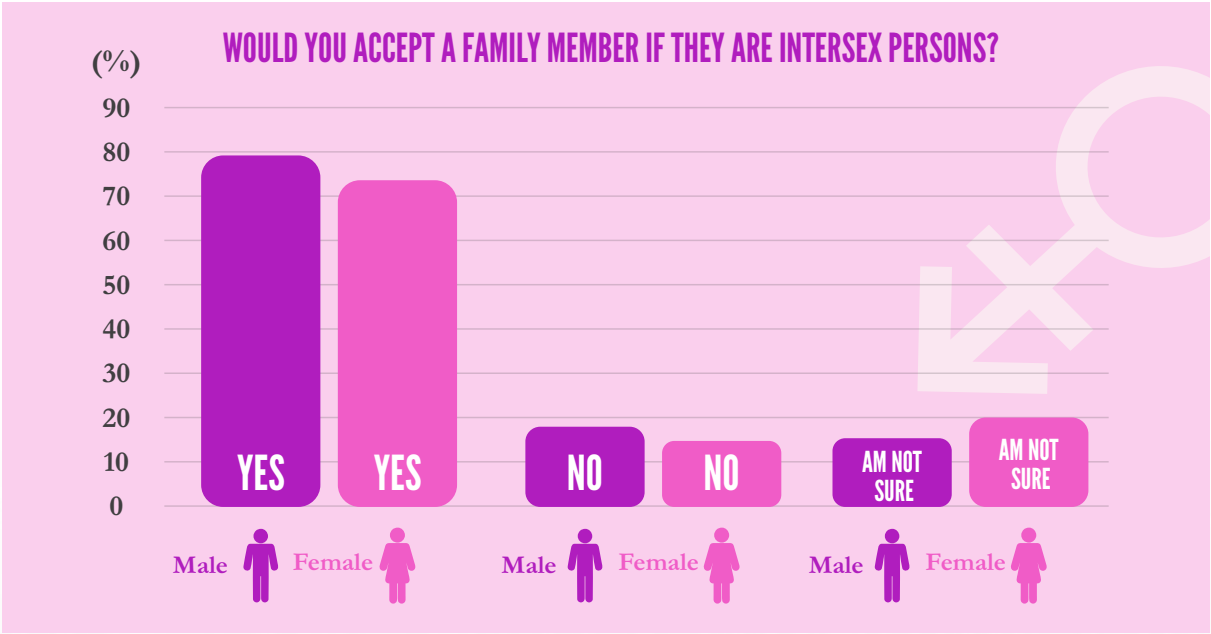


Figure 7i. Acceptance of intersex persons among family members.

Further analysis based on age groups reveals interesting patterns that shed light on how age influences perception, discrimination, and potential violence. The youngest group (under 18) shows a generally positive outlook, with 73% willing to accept an intersex family member, while 20% remain unsure. This highlights a need for increased education on the topic. In contrast, the 35-44 age group demonstrates the highest acceptance at 81%, suggesting a greater openness to diversity. Meanwhile, the 45-54 age group exhibits the lowest acceptance at 63%, with 21% explicitly stating they would not accept an intersex family member.

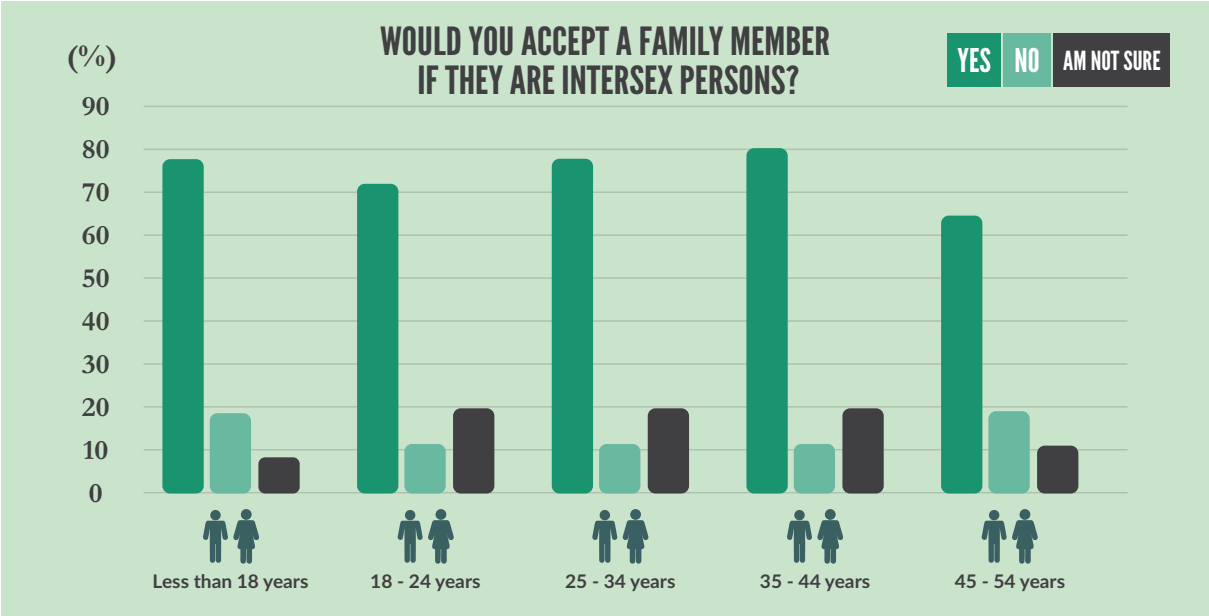


Figure 7ii. Acceptance of intersex persons among family members.

ii. Perception of Intersex Individual

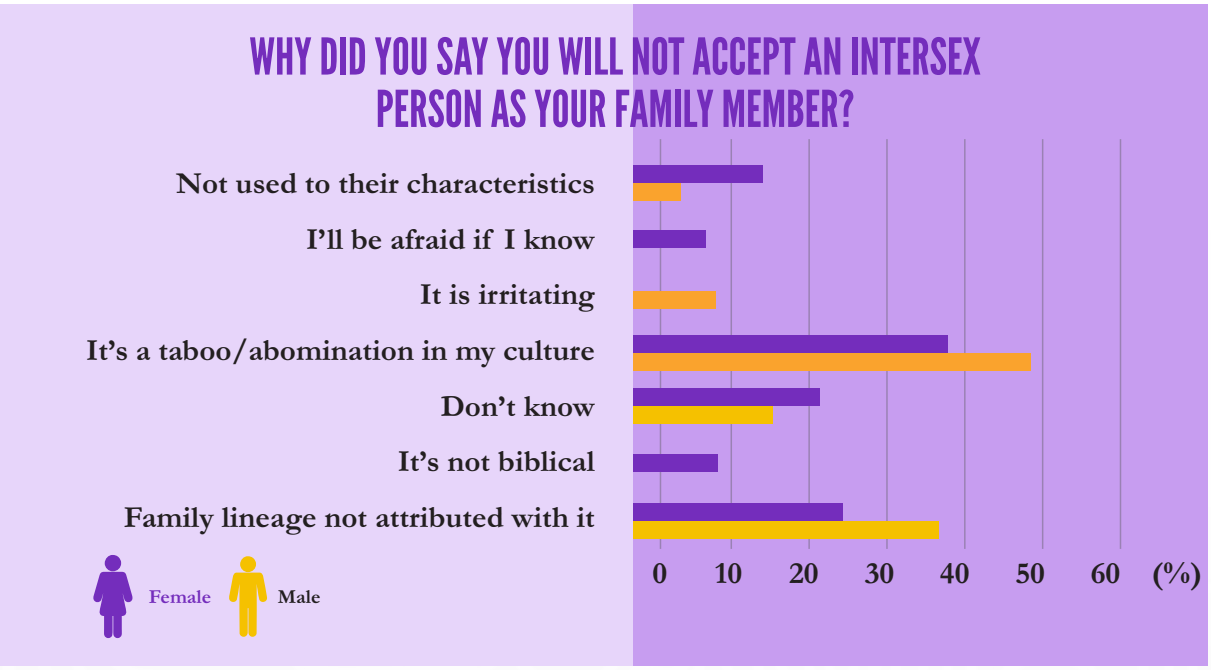


Figure 8: Responses on Perception of intersex

In figure 8. the data on why respondents would not accept an intersex family member highlights several key factors contributing to this resistance. The most prominent reason cited by 16% of respondents is the fear of stigma associated with accepting an intersex family member, indicating the influence of societal perceptions and potential negative reactions from others. Additionally, 11% expressed uncertainty, revealing a lack of understanding or awareness about intersex. Furthermore, 11% view intersex as taboo or an abomination, while 7% label it as abnormal, reflecting deep-rooted cultural or societal beliefs that need to be challenged. Lastly, 2% mention religious reasons, deeming intersex unbiblical.

iii. Recognition of Intersex Persons

Figure 9 data reveals a concerning lack of recognition for intersex individuals in Nigeria, with only 16% of respondents believing they receive enough recognition, while 84% disagree. Gender differences in perception are minimal, with both male and female respondents expressing low levels of awareness. Among the youngest age group (under 18), only 13% believe intersex individuals receive sufficient recognition, while older age groups show slightly higher percentages but remain low overall. Even in the 45-54 age group, where the highest percentage (32%) believe intersex individuals receive enough recognition, the majority (68%) still disagree. These findings underscore the urgent need for increased awareness and education on intersex to foster greater recognition and understanding of the intersex community in Nigeria across all age groups and genders.

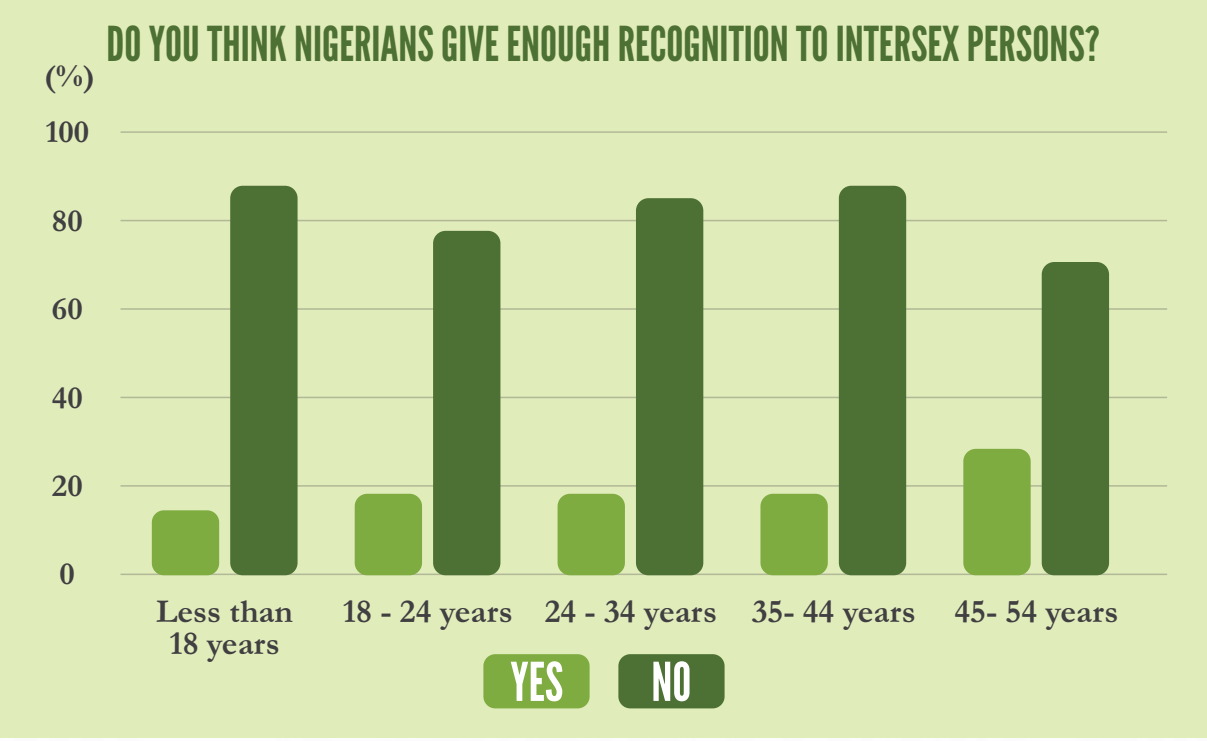
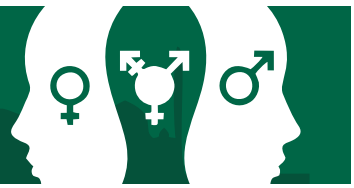


Figure 9: Responses on recognition of Intersex

iv. Discrimination and Violence Against Intersex Persons in Nigeria

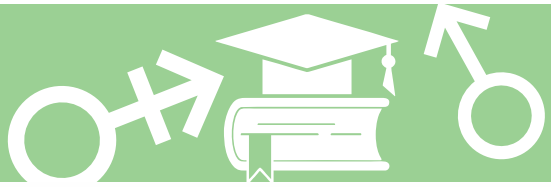
The responses from the survey provide valuable insights into the perceptions of violence and discrimination experienced by intersex people in Nigeria. Overall, 31% of the respondents stated that intersex individuals do experience violence and discrimination, while the majority, 69%, believe they do not. This indicates that a significant portion of the population acknowledges the challenges faced by intersex individuals in Nigeria, while a larger majority do not. 28% of male respondents believe intersex people face discrimination and violence, while 72% disagree. On the other hand, female respondents are slightly more likely to perceive such experiences, with 34% acknowledging discrimination and violence, and 66% expressing the opposite view.

4.1.2 Situation of Intersex persons in Nigeria



Most of the interviewees agree that it is important to amplify intersex voices and visibility to establish in the perception of society that intersex persons are people with equal rights and should be free from discrimination and stigmatisation. This section draws insights from the focus group discussion held with intersex respondents and parents of intersex children and it highlights the lived experience of intersex persons in Nigeria, exploring the challenges they face including death and abandonment, lack of legal recognition, discrimination, lack of legal framework, coerced, uninformed and unnecessary genital normalizing surgeries on minors aimed at altering the sexual and reproductive anatomy to suit social classifications of male and female.

I. Access to Education



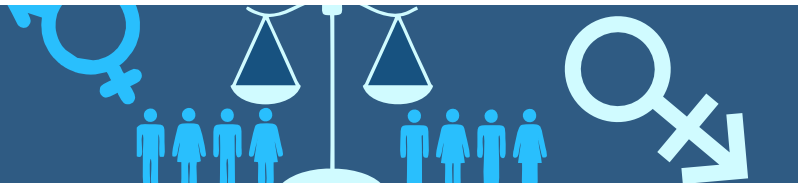
Baseline survey found that a majority of the intersex persons interviewed reported that they were either forced to drop out of school or their education was hindered due to negative peer pressure and societal stereotyping, leading to stigma and discrimination from students and staff associated with their physical development during puberty.

"Being an intersex person affected my ability to get a good education. I grew up in the north and so I was considered an abomination. I was unable to go to school because I had to consistently explain myself to both students and faculty— it has taken me 7 years to finally get around to my Bachelor's degree, this affected my ability to find work too."

Furthermore, they reported facing problems in accessing sanitation, including toilets, showers and changing rooms as students, especially in boarding houses and in higher education school hostels.

School curricula are non-inclusive of the reality and the lived experience of intersex persons and counselling services are simply unavailable, especially in secondary schools. In fact, Nigeria has categorically removed comprehensive sexuality education from its improved education strategy for primary and secondary schools further making it impossible for students to have access to education on sexual orientations and gender identity.[23] During the 66th National Council on Education (NCE) meeting in Abuja, themed "Strengthening of Security and Safety in Nigerian Schools for the Achievement of Education 2030 Agenda," Nigeria's then Minister of Education, openly disclosed that upon discovering the inclusion of sex education in the basic education curriculum, he promptly engaged with the Nigerian Educational Research and Development Council (NERDC) Executive Secretary to ensure its removal.[23]

ii. Access to Justice



Access to justice for intersex persons in Nigeria remains a significant challenge due to the lack of legal recognition and specific protections for their rights. Without explicit legal provisions, intersex individuals are forced to face discrimination, violence, and other human rights abuses on their own. The legal system inadequately addresses their unique needs and concerns, making it difficult to near impossible to access justice when their rights are violated, both in public spaces and within their own families and communities.

Respondents say that even though they recognize that every Nigerian has fundamental human rights under Chapter 4 of the Constitution of the Federal Republic of Nigeria, these rights do not accrue to intersex persons because society continues to violate them, and there is almost never justice served. They agree that the violation of bodily autonomy is central to the violations experienced by intersex persons in Nigeria. For example, intersex persons are arrested indiscriminately by the police for their choice of expression.

"Justice does not exist for intersex persons in Nigeria. It is like you are invisible. I have been harassed by the police several times for not looking like the gender on my id, they arrest you and after torturing you, they still take the small change you may have on you."

Additionally, the absence of legal recognition of intersex individuals in official documents such as birth certificates result in difficulties when accessing justice. As

Nigeria's court system is primarily binary-oriented, intersex persons face challenges related to their gender identity during legal proceedings, leading to misgendering, violence and further marginalization within the justice system.
Furthermore, the general lack of awareness and understanding of intersex variations within Nigeria's legal system contributes to this systemic discrimination. It extends to law enforcement officials, lawyers, and judges, making it harder for intersex individuals to receive fair treatment and understanding in legal matters.

iii. Access to Healthcare



A major challenge in the lived experience of intersex persons in Nigeria is the mistreatment they receive from medical institutions in the country. Intersex persons polled all alluded to the lack of privacy and general disdain with which they are treated.

A respondent reflected on their experience:
"When the nurse noticed that I presented as male but the medical issues I presented with were representative of the hormonal challenges experienced by women, she went to call other colleagues and they talked over me like I was not there, discussing my identity with such contempt. I felt violated."

Respondents also allude to unnecessary and invasive medical examinations as a major challenge. Intersex persons are treated as lab rats for experiments and are put through unnecessary medical examinations even when they present with symptoms that do not relate to their variations in sex characteristics.

Another Respondent reflects:
"I didn't know that wasn't how I should be treated. The doctors would ask me to undress at every appointment and doctors would call on other persons to come in and they would take pictures of my genitals. When I go for simple appointments such as medicine pick up, medical practitioners who have heard about my situation would ask me to undress in the name of medical examination. I found out this was wrong at a later time and that made me stop going to the hospital even when I am ill."

This report identified an intersection in the right to health and education as intersex children that are sent to school have to interact with school nurses and doctors who do not possess adequate education and information on the intersex condition. This compounds the difficulty intersex children experience in schools and directly impacts their right to education. Here, another respondent reflects—

"When I was in secondary school, I would have to go home every time I fell ill— I have had cases in which the school nurse would administer injections for malaria even though I knew I did not have malaria."

The surveyed demographic consistently referenced expensive and unaffordable diagnosis. The diagnosis of intersex variations often involves comprehensive genetic and hormone testing, imaging, and consultations with specialists. These procedures can be expensive and are usually not covered by insurance — more importantly, the availability of insurance options is not widespread in Nigeria especially among marginalized communities.

Emotional and mental health challenges have been identified as a Cardinal Health issue faced by intersex persons. In grappling with issues surrounding their gender identity and sexual orientation, intersex persons do not have adequate access to counselling and support to help navigate these complexities. Respondents face emotional and psychological challenges, especially when dealing with societal stigma and misunderstandings. Access to mental health support is crucial for their well-being. Furthermore, the management of identity issues resulting from diagnosis is an important factor. Non-consensual medical interventions, such as unnecessary surgeries performed to conform to societal norms leave children traumatized, and treating and healing from such trauma requires specialized care.

Parents of intersex children try to make informed decisions regarding their treatment, acceptance in society, access to education and most importantly access to informed healthcare. They are usually responsible for the decision whether the child will undergo surgery. This responsibility comes with confusion regarding the medical urgency of the surgery. This burden is coupled with the fact that the medical environment is not free from cultural biases and current medical protocols and societal treatment of intersex people are often based on false stereotypes about gender.

Respondents (parents of intersex children) experience challenges in taking informed decisions for their wards, including challenges with regards access to specialist healthcare professionals, medical cost, and the pressure to allow surgical interventions. Respondents allude to the fact that the challenges include surgery without proper understanding of the different ramifications, multiple surgeries with dire consequences, and lack of support during the treatment for both the child and themselves. All the respondents have negative experiences with the treatment of their intersex child.

There is little or no support from health care professionals and social support from the government. The only support we have are from family and friends.

Surgeries include removal of clitoral tissue considered overlarge; reconfiguration of penises with "openings" other than at their tip; removal of ovarian or testicular tissue diagnosed as inappropriate to the sex assigned etc. These procedures may sound reassuring and a

correction of 'errors' but they entail numerous problems including long-term psychological and physical outcomes.

The parents of intersex children interviewed express their fear, rage and despair at the uncertainties of the surgeries, the failed promises of medical personnel who assure them that the surgeries will be one time and they are now faced with multiple unending surgeries.

I hope I am making the right decision for my child because the surgery is irreversible.

iv. Access to Legal Recognition

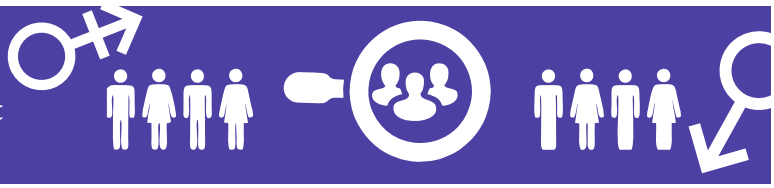


The lack of legal recognition lays the groundwork for the systemic exclusion of intersex persons from educational institutions, and employment opportunities while also exposing them to police violence. Specifically, intersex persons who are diagnosed as adults may need to change their gender identities to match their genetic composition and this renders their identity unrecognized by law. They are unable to receive certificates, get jobs and are pushed to the margins of society. A respondent reflects on the struggle for legal recognition.

"I have not been able to receive my NYSC certificate. I was told to go to the court to get an affidavit. This process has been incredibly hard because until I am able to achieve legal recognition, name change and a corresponding gender assignment, I am excluded from working to make a living. I have had to do jobs that do not require a certificate to keep body and soul together, but I am seriously underemployed and need a solution."

Without specific legal provisions to safeguard their rights, intersex individuals are left without adequate recourse to seek justice or protection from harm. This contributes to a climate where perpetrators may act with impunity.

v. Access to Employment



From extensive discussions with respondents who identify as intersex, it is evident that access to employment in Nigeria is challenging due to widespread discrimination and societal stigma surrounding intersex variations. There are no workplace policies and legal protections addressing the rights of intersex individuals. As a result, intersex individuals face barriers when seeking employment and experience discrimination during the hiring process or while on the job. Discrimination in the workplace against intersex persons in Nigeria takes various forms, including direct rejection during job applications, unequal treatment in terms of job assignments, promotions, or benefits, and even harassment from coworkers or employers. Employers largely hold

misconceptions or biases about intersex individuals, leading to exclusion from job opportunities based on their intersex status rather than their qualifications and abilities.

"I am an intersex person who identifies as female but have male features and I have to wear a disguise to work every day to convince my job that I am a man even with my disguise, they still suspect me."
Societal stigma and lack of awareness about intersex variations contribute to a hostile work environment for intersex employees. This may cause them to hide their identity for fear of discrimination or backlash, leading to a sense of isolation and emotional distress. As a result, intersex persons in Nigeria may encounter unique challenges in finding and maintaining gainful employment.

"I was unable to use the locker room at my former job, so I have to commute back home in the same clothes I work in, and my house is very far. The difficulty was so much so I changed my job, but I also had the same challenges at my new job. I cannot hide."

vii. Socio-Cultural Discrimination and Exclusion



Social and cultural discrimination, exclusion, and violence against intersex persons in Nigeria are deeply rooted in various aspects of the society. Mythical, cultural, and religious beliefs label intersex persons as unnatural and an abomination. Intersex individuals are born with sex characteristics that do not fit typical definitions of male or female, and they often face significant challenges in Nigeria due to prevailing attitudes and beliefs. A lack of awareness, ignorance, and lack of information about intersex variations solidifies these socio-cultural beliefs that contribute to the exclusion and fetishization of intersex persons, giving rise to sexual violence and other forms of sexual harassment against them.

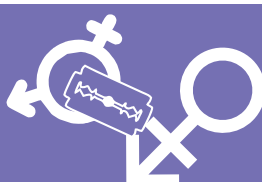
"People are ignorant and unaware of the intersex variations. So many people come to my DMs trying to ask me about my body and how I have sex. Some of them try to use me to try a new thing, like an experiment."

Intersex variations challenge the binary view of gender historically adopted in Nigeria and leads to confusion and discomfort for some individuals who may not understand or accept the existence of intersex persons. Unfortunately, these discomforts are expressed through stigmatisation and shame. Intersex variations are sometimes viewed as abnormal or unnatural in religious and cultural circles and this stigma leads to shame and secrecy within families, making it difficult for intersex individuals to openly express their identities or seek support and live a life they enjoy.

"Many families hide their intersex children. They look at them as "the black sheep," like a child that should not be here, so they isolate them. This causes emotional, mental, and physical damage to people. I've met people who cannot speak or relate to others in public because of years of isolation."

The key informant conversations provided a deep and personal perspective on intersex identities and how they navigate simple societal structures that are otherwise taken for granted. We believe that to truly bring forward a study that calls us to action, these personal perspectives from intersex persons, must take center stage. James Baldwin encourages, 'Take no one's word for anything but trust your experiences. We choose to trust the voices of intersex persons in Nigeria, and we honor their experiences as valid sites where change will begin.

4.2. Public Perception of Intersex Genital Mutilation (IGM)



The results in Figure 10 indicate that there is a concerning lack of understanding regarding intersex genital mutilation (IGM) (*Surgical, hormonal, and other medical interventions performed to modify atypical or ambiguous genitalia and other sex characteristics, primarily to make a person's appearance more typically male or female. Note that these medical interventions most times have severe health and mental health consequences*) among the respondents. While 80% claim to know what IGM is, the percentages suggesting incorrect definitions are quite significant. Only 15% correctly identify it as a medical correction surgery, while 2% believe it is a mode of sterilization and 5% claim it

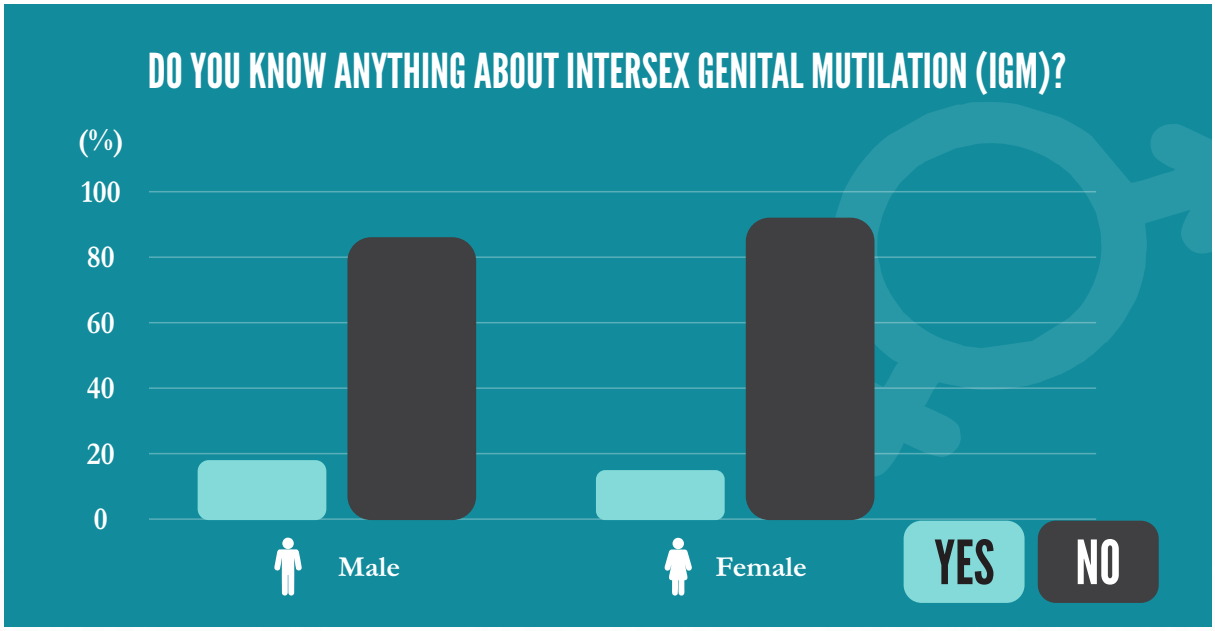


Figure 10: Responses on Knowledge of IGM

The low percentage (15%) of respondents who correctly identified IGM as a medical correction surgery is alarming. This lack of awareness highlights a potential gap in knowledge regarding intersex issues, particularly in the context of medical interventions. It is essential to address this lack of understanding as it can perpetuate harmful practices and misconceptions related to intersex individuals' medical needs. Furthermore, 5% of respondents claiming that IGM involves the removal of genitals is particularly concerning.

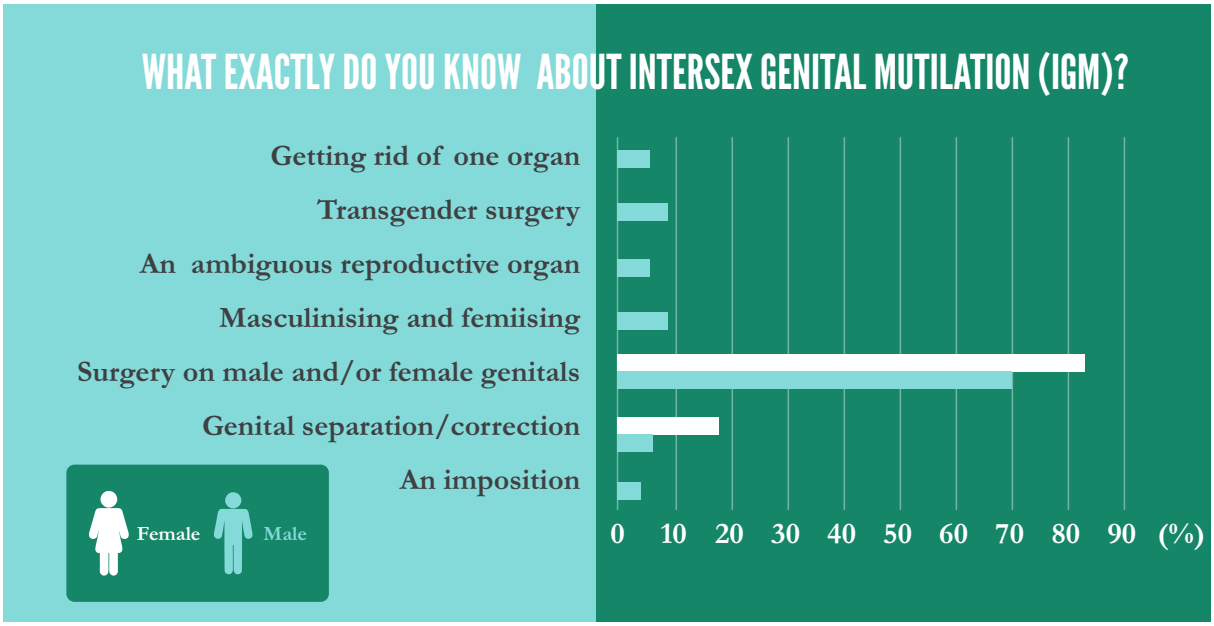


Figure 11: Responses on knowledge of IGM specifics.

4.3. Legal Framework Protecting Intersex Persons from IGM

The results regarding respondents' knowledge of legal frameworks addressing IGM in Nigeria indicate a significant lack of awareness and uncertainty among the respondents. A small minority (12%) claim to be aware of existing laws that address IGM, while the majority (52%) express uncertainty about the presence of such legal frameworks. A proportion (37%) claim that there are no legal measures in place to address intersex genital mutilation in Nigeria.

This suggests a lack of access to information or limited awareness about IGM and related legal protections in the country. This lack of clarity may contribute to a lack of action or advocacy on the issue. Additionally, the number of respondents asserting the presence of legal measures indicates that there may be a misconception or a perception about IGM because Nigeria currently does not have a legal framework addressing IGM.

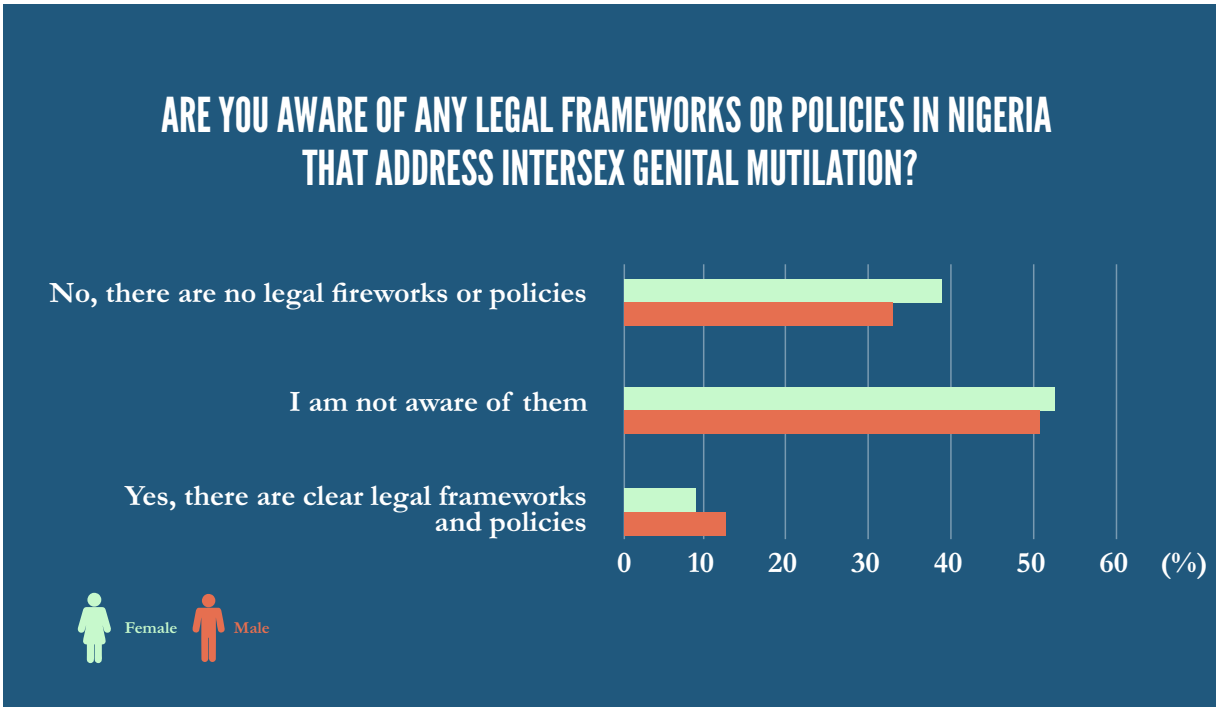


Figure 12: Responses on policies to address IGM.

4.3.1. Awareness of Challenges Faced by Intersex Persons in Nigeria

The poll results suggest that awareness of the challenges faced by intersex persons in their daily lives is relatively low among the respondents. Only 32% of the total respondents claim to be aware of the challenges, while the majority, 68%, said they are not aware of any challenges. This indicates a significant knowledge gap regarding the experiences and difficulties that intersex individuals may encounter in their daily lives.

When examining the results by gender, the patterns remain consistent. Both male and female respondents show a similar level of awareness, with 32% of men and 33% of women claiming to know about the challenges faced by intersex individuals. The majority of both genders, 68% of men and 67% of women stated that they are not aware of these challenges. These findings suggest that there is no substantial difference in awareness between men and women regarding struggles experienced by intersex individuals.

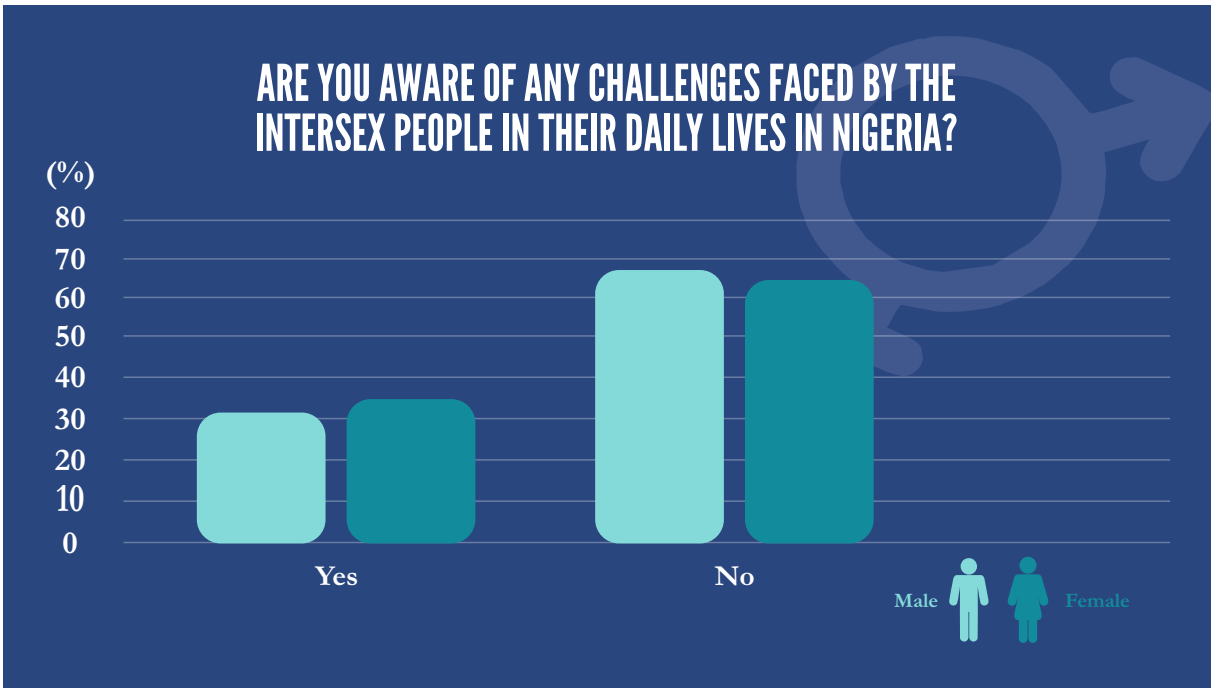


Figure 13: Responses on awareness of challenges faced by intersex.

4.4. Perception of Intersex Genital Mutilation in Nigeria

A comprehensive perception survey was conducted across all six geopolitical zones of Nigeria to gain insights into societal attitudes and perceptions of Intersex Genital Mutilation (IGM). The survey collected responses from 1000 participants, with an equal distribution of 505 male and 495 female respondents. It aimed to explore the prevalence, extent, and scope of IGM in Nigeria, gathering diverse perspectives from individuals representing different demographics.

The respondents were distributed across various age groups, religious affiliations, and geographical regions in Nigeria. The survey encompassed youth (21% below the age of 18), young adults (34% aged 18-24), individuals in their prime working years (28% aged 25-34, and 13% aged 35-44), and more experienced segments of society (4% aged 45-54). This diverse age distribution contributed to a comprehensive understanding across different generations in Nigeria regarding IGM practices.

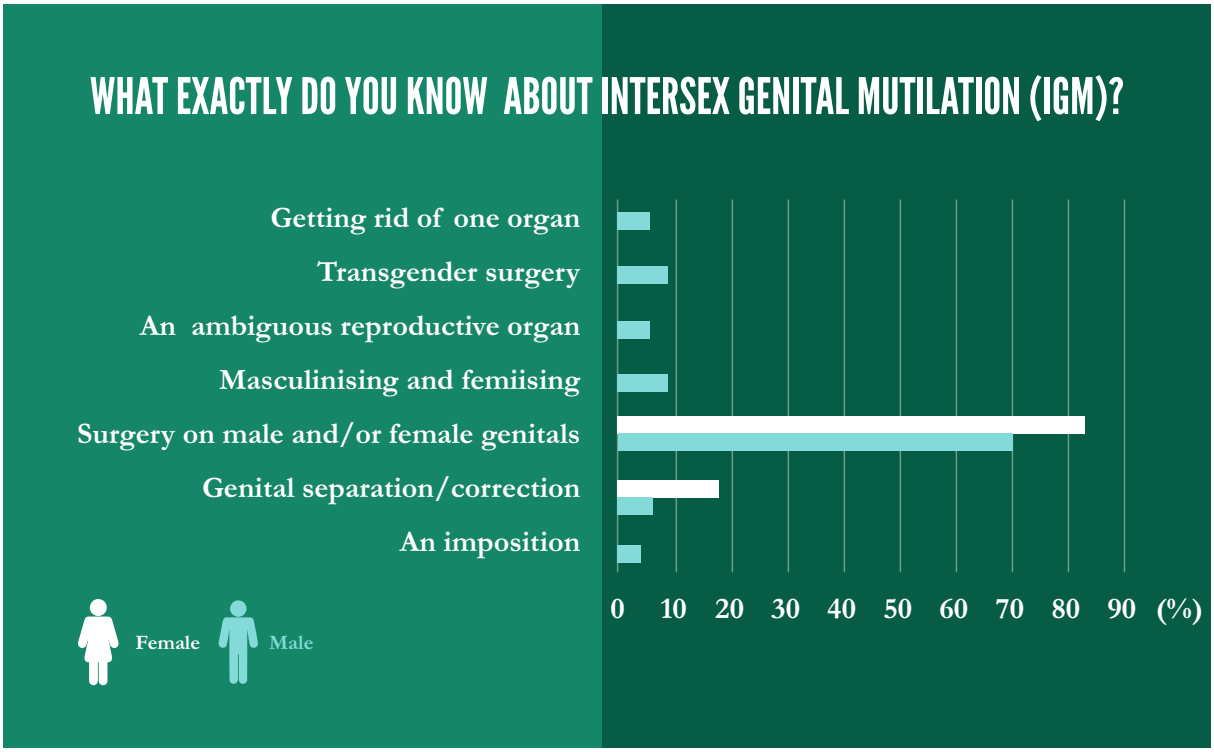


Figure 14: Responses on IGM perception

4.4.1. Personal Knowledge of IGM Survivors

The data on personal knowledge of IGM survivors among the respondents reveals some interesting patterns. Overall, only 12% of the total poll claimed to have personal knowledge of IGM survivors, while the majority, 78%, said they did not, and 10% were unsure.

It appears that a higher percentage of women (22%) claim to have personal knowledge of IGM survivors compared to men (5%). This difference might be attributed to various factors, including women being more likely to have personal connections or access to family information about intersex issues and IGM survivors.

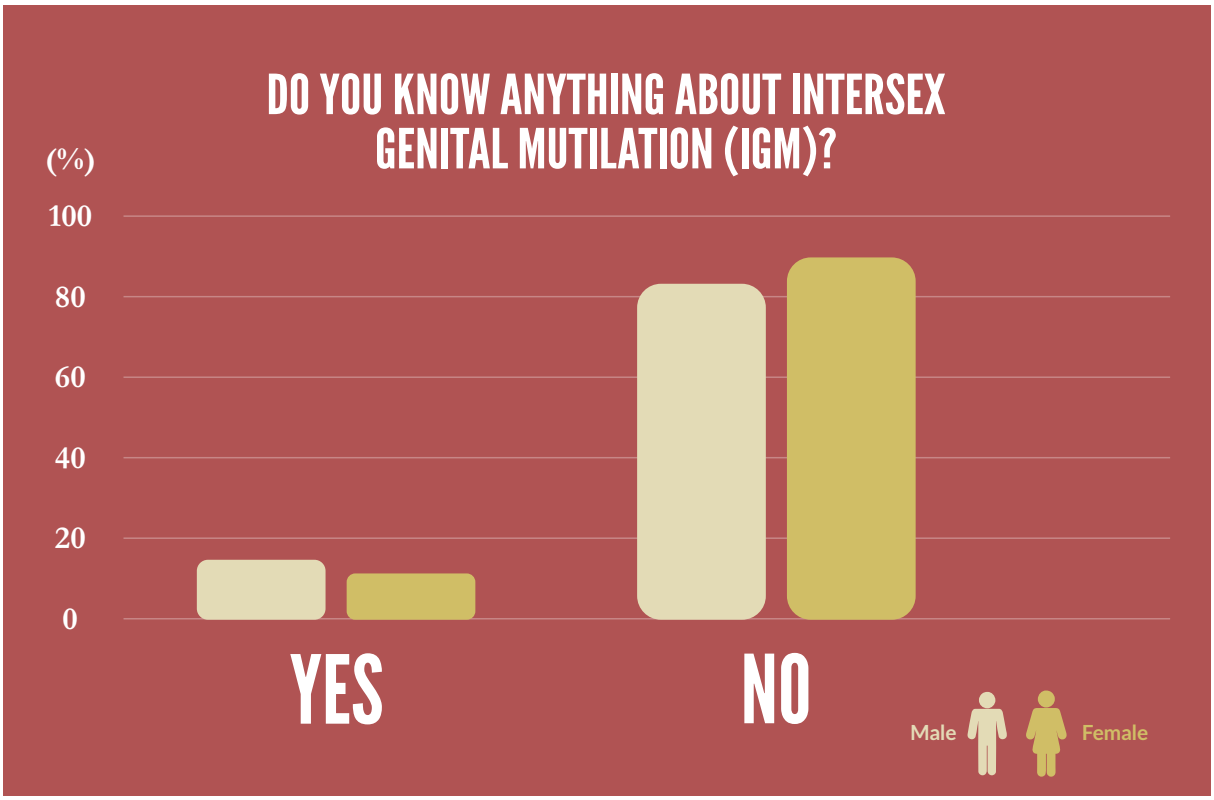


Figure 15: Knowledge on IGM survivors

Examining the data by age groups as seen in figure 15, some trends emerge. Respondents between the ages of 18-24 and 35-44 both have 10% claiming to have personal knowledge of IGM survivors, while the majority in both age groups (75% for 18-24 and 80% for 35-44) deny having such knowledge. The 25-34 age group has a slightly higher percentage (16%) of respondents claiming personal knowledge, but the majority (80%) still do not have such knowledge.

Interestingly, respondents in the 45-54 age group reported 0% claiming personal knowledge, with 50% unsure and the remaining 50% denying any knowledge. This suggests that awareness and understanding of IGM survivors' experiences may vary across different age groups.

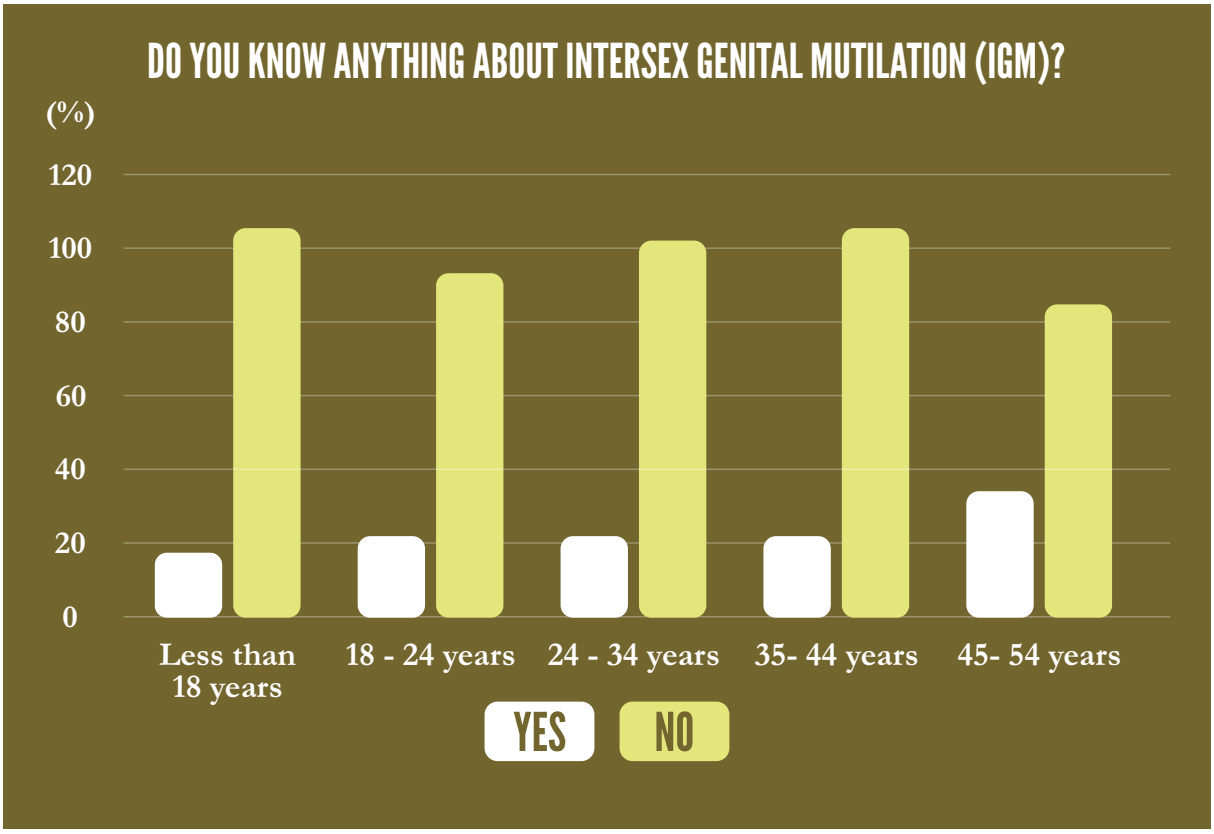


Figure 16: Knowledge on IGM by age group

4.4.2. Public Perception of Legal Frameworks Protecting Intersex Persons from IGM

Regarding respondents' knowledge of legal frameworks addressing IGM in Nigeria, there is indication of uncertainty and a significant lack of awareness among the respondents. A small minority (12%) claim to be aware of existing laws that address IGM, while the majority (52%) express uncertainty about the presence of such legal frameworks. A proportion (37%) claim that there are no legal measures in place to address intersex genital mutilation in Nigeria.

CONCLUSION

The visibility, inclusion, and recognition of intersex persons in Nigeria is a complex discourse which requires urgent and holistic approaches to address the challenges faced by these often marginalized persons in Nigeria. Throughout this research, we have uncovered a range of issues including limited access to healthcare, legal recognition, employment, housing, education, and fundamental human rights. We have also explored the scope, extent, and prevalence of IGM and uncovered public perception — myths and misconceptions on intersex variations in Nigeria.

The practice of intersex genital mutilation has been highlighted as a challenge to the lives and well-being of intersex persons in Nigeria. This harmful practice not only violates human rights but also exacerbates the physical and psychological trauma experienced by intersex individuals. Eradicating intersex genital mutilation is a critical step towards promoting the well-being and rights of intersex persons in Nigeria.

A comprehensive and coordinated approach is essential to address the myriad challenges facing intersex individuals in Nigeria. Government agencies, civil society organizations, healthcare providers, educators, and the wider society must collaborate to promote the visibility, inclusion, and recognition of intersex persons. Only through a holistic approach that addresses healthcare, education, housing, and fundamental rights, while also ending IGM, can we aspire to create a society where intersex individuals can live their lives with dignity, free from discrimination, and with equal opportunities. This study is a call to action to prioritise the holistic well-being and rights of intersex persons in Nigeria.

RECOMMENDATIONS

Government and Policy Makers



- Develop legal frameworks that safeguard the rights and autonomy of intersex persons, particularly policies and guidelines (including clinical guidelines) that protect and promote the rights of intersex persons, intersex children and their parents from uninformed and unnecessary corrective surgery.

- The government at the federal and state levels must ensure that policies are stipulating that surgery should be delayed where a child is born with ambiguous genitalia if there are no medical conditions needing immediate attention or emergencies.

- Establish a dedicated unit or desk in appropriate ministries and parastatals for social workers who are trained to provide the necessary and appropriate advice, support (including psychosocial support) and resources to intersex children and their parents especially at birth.

- Provide special healthcare facilities and counselling services for intersex children and their families. This is essential to address the unique medical needs of intersex children and their families.

- Provide for easy administrative processes which make it easy for intersex persons to amend name and sex on a birth certificate or any other legal document.

- Investigate and monitor human rights violations against intersex persons through national human rights institutions and ensure that victims of such human rights violations have access to justice and reparations.

- Ensure that all intersex persons and their families receive state-supported psych-social support to the maximum of its available resources.

- Collate disaggregated data on the circumstances of intersex persons in Nigeria and provide a comprehensive care infrastructure.

- The government must respect the right to autonomy without discrimination based on real or perceived sexual orientation and gender identity of intersex persons.

- Prohibit discrimination of intersex persons in education, health care, employment, sports and access to public services, and address such discrimination through enacting laws and policies, reviewing existing policies, and repealing any laws to ensure that the rights of intersex persons are protected.

- Restructure the system that emphasizes the sex/gender requirement on birth certificates to enable intersex persons to register without falling into the two binaries of male and female.

Health Professionals and Social Workers



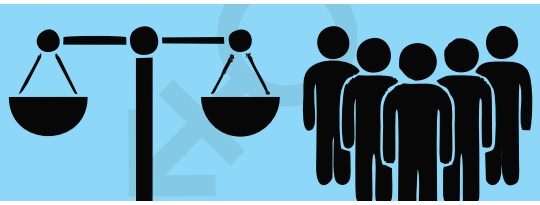
- Healthcare professionals and social workers should be at the forefront of advocacy for services for intersex individuals and their families. Participants shared the need to make the issue of intersex individuals public by lobbying lawmakers for protection.

- There is a need for capacity building and awareness-raising for healthcare professionals and social workers on intersex, the adverse implications of IGM and other surgically unnecessary procedures that aim to 'correct', including compulsory training on sex characteristics and all intersex variants for medical and social workers in Nigeria.

- In-depth training is necessary for health professionals and social workers on sex characteristics, and caring for intersex persons including medical etiquette, human rights and sexual orientation, gender identity and expression.

- Ensure that all intersex persons and their families receive state-supported psycho-social support to the maximum of its available resources.

Civil Society and intersex rights activist



- Continue to advocate against discrimination of intersex persons in education, health care, employment, sports, and access to public services, and address such discrimination.

- There is a need for civil society and intersex rights activist to understand that a holistic approach is required to address the challenges facing intersex persons in Nigeria. These approaches must include holding the government accountable while influencing social structures such as religious and cultural spaces.

- Continue to advocate for comprehensive sexuality education to expose the upcoming generations to the true diversity of our world and break free from white supremacist binaries of sex and gender.

- Continue to train and support medical professionals on how to properly address the needs of intersex persons and immediate family members without violating the rights of the concerned persons.

REFERENCES

1. https://www.chr.up.ac.za/the-intersex-project
2. https://www.unswlawjournal.unsw.edu.au/article/fixed-at-birth-medical-and- legalerasures-ofintersex-variations
3. https://ihra.org.au/wp-content/uploads/key/Intersex-inclusion.pdf
4. https://www.chr.up.ac.za/images/researchunits/sogie/documents/ Intersex_Fact_Sheet.pdf
5. https://www.reuters.com/article/us-kenya-lgbt-intersex-trfn-idUSKBN1XE1U9
6. https://book.coe.int/en/commissioner-for-human-rights/6683-pdf-human-rights-andintersex-people.html
7. https://intersexnigeria.org/resources/
8. https://ng.boell.org/en/2021/07/06/anyone-can-be-born-intersex-photo-portrait-storyintersexnigeria
9. https://www.unswlawjournal.unsw.edu.au/wp-content/uploads/2017/09/39-2-3.pdf
10. https://achpr.au.int/en/adopted-resolutions/resolution-promotion-and-protection-rightsintersex -persons
11. https://www.constituteproject.org/constitution/Nigeria_2011
12. https://www.ohchr.org/sites/default/files/Documents/Publications/ Compilation1.1en.pdf
13. https://en.wikipedia.org/wiki/Intersex_rights_in_South_Africa
14. https://www.knchr.org/our-work/special-interest-groups/intersex-persons-in kenya#:~:text=Article%2027%20of%20the%20Constitution, Rights%20as%20any%20ot her%20person.
15. https://humanrights.gov.au/our-work/employers/sexual-orientation-gender-identity-andinterse x-status-discrimination
16. Tsola Bassey Ekpokai (2017) INTERSEXUALITY AND DECISION MAKING OF NIGERIAN PARENTS A Thesis Presented to the Faculty of California State University, Stanislaus. https://scholarworks.calstate.edu/downloads/wh246s89w
17. As above.
18. Mavhandu-Mudzusi, Azwihangwisi. (2017). Impact of stigma and discrimination on sexual wellbeing of LGBTI students in a South African rural university. South African Journal of Higher Education. 10.20853/31-4-894.
19. The birth of an intersex child is emotionally traumatizing for parents (Nigerian) because the child's sexual ambiguity "violates" a deeply held socio cultural worldview, and because it brings out feelings of shame and guilt (Baqutayan, 2014). https://journalarrb.com/index.php/ARRB/article/download/25525/47748/

20. Shadiya Mohamed & Saleh Baqutayan 'The Psychological Distress and Coping Mechanisms among Intersexed Individuals" (2014) Annual Research & Review in Biology 4(19) SCIENCEDOMAIN international www.sciencedomain.org
21. Purohit and Another v The Gambia (2003) AHRLR 96 (ACHPR 2003)
22. https://achpr.au.int/en/adopted-resolutions/resolution-promotion-and-protection-rightsintersex -persons
23. Removal of sex education from curriculum: Is Nigeria scoring another own goal? https://guardian.ng/features/removal-of-sex-education-from-curriculum-is-nigeriascoring-another-own-goal/
24 https://gams.be/en/intersex-genital-mutilation/
T. Ekpokai, 'Intersexuaity and Decision Making of Nigerian Parents', https://scholarworks.calstate.edu/concern/theses/7h149q77f?locale=en , See also A. Fausto-Sterling, 'The five sexes', (1993) *The Sciences*, 33(2), 20-24; C. Migeon *et al*, 'Ambiguous genitalia with perineoscrotal hypospadias in 46, xy individuals: Long-term medical, surgical, and psychosexual outcome' (2002) *Pediatrics*, 110(3), 1-10.
25 A. Fausto-Sterling, 'The five sexes', (1993) *The Sciences*, 33(2), 20-24; see also . Ekpokai, 'Intersexuaity and Decision Making of Nigerian Parents', https://scholarworks.calstate.edu/concern/theses/7h149q77f?locale=en
26. https://scholarwoBall, H. L., & Hill, C. M. (1996). Reevaluating" twin infanticide". Current Anthropology, 37(5), 856-863.rks.calstate.edu/concern/theses/7h149q77f?locale=en
27 A. Puluka, (2015) 'Parent versus state: Protecting intersex children from cosmetic genital surgery' *Michigan State Law Review*, 2015(5), 2095-2141.